

Behavioral Activation for Treatment-resistant Depression: Theoretical Model and Intervention

Protocol

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Abstract

Treatment-Resistant Depression (TRD) constitutes a severe public health problem and is a condition scarcely addressed by psychological therapies. This paper presents a theoretical model, grounded in established learning principles and in the perspective of Behavioral Activation (BA), to explain its constitution and development. The model reflects how patients with TRD are more susceptible to becoming trapped in their condition by seeking to avoid discomfort through avoidance and escape behaviors, which increasingly distances them from sources of positive reinforcement. Based on this model, a BA-based intervention protocol is proposed for the treatment of TRD. Through six phases (in a total of thirteen sessions), the protocol guides the intervention towards the reestablishment of personalized routines to increase the probability of reinforcement and reduce avoidance behaviors. Future research will allow the evaluation of the efficacy of the protocol as a standalone intervention or in conjunction with pharmacological treatment.

Key Words: Behavioral activation; Treatment-resistant Depression; Treatment protocol; Major Depressive Disorder; Treatment-resistant Depression Theory.

Resumen

La Depresión Resistente al Tratamiento (TRD) constituye un severo problema de salud pública y es una condición poco abordada por las terapias psicológicas. En este artículo se presenta un modelo teórico, fundamentado en contrastados principios de aprendizaje y en la perspectiva de la Activación Conductual (AC), para explicar su constitución y desarrollo. El modelo refleja cómo los pacientes con TRD son más susceptibles a quedar atrapados en su condición al procurar evitar el malestar por medio de comportamientos de evitación y escape, lo que les aleja cada vez más de las fuentes de

reforzamiento positivo. A partir de este modelo, se propone un protocolo de intervención basado en la AC para el tratamiento de la TRD. A través de seis fases (en un total de trece sesiones) el protocolo guía la intervención hacia el restablecimiento de rutinas personalizadas para incrementar la probabilidad del refuerzo y reducir los comportamientos de evitación. Futuras investigaciones permitirán evaluar la eficacia del protocolo como intervención única o en unión con el tratamiento farmacológico.

Palabras clave: Activación conductual; Depresión resistente al tratamiento; Protocolo de tratamiento; Trastorno depresivo mayor; Teoría de la depresión resistente al tratamiento.

The term Treatment-resistant Depression (TRD) denotes individuals diagnosed with Major Depressive Disorder (MDD) who exhibit no improvement following prescribed pharmacological interventions (Fawcett & Kravitz, 1985; Heimann, 1974). TRD represents a complex clinical entity comprising various subtypes unresponsive to treatment. It is widely recognized as among the most challenging clinical manifestations of depressive disorders, leading to substantial treatment costs. Moreover, TRD patients are twice as likely to require hospitalization, incurring hospitalization costs six times higher than the average for non-TRD patients. Furthermore, TRD is associated with poorer long-term outcomes, including functional decline, heightened risk of relapse, and increased incidence of suicidal attempts compared to those achieving complete remission (Corrales et al., 2020).

At present, there is no universally accepted consensus regarding the criteria for identifying TRD. The prevailing conceptualization of TRD is pharmacological and involves the lack of response to two or more trials of monotherapy with distinct antidepressants, each administered separately at an appropriate dosage over a defined duration (Berlin & Turecki, 2010).

Psychotherapy in Treatment-Resistant Depression

While research findings do not unequivocally endorse psychotherapy as a standalone intervention for TRD, it is widely acknowledged that the adjunct of personalized depression-focused psychotherapy to pharmacotherapy may increase the probability of superior outcomes (Álvarez et al., 2008; Spijker et al., 2013). In fact, it has become increasingly recognized that addressing TRD necessitates the formulation of a psychotherapeutic plan in conjunction with pharmacological treatment, to the extent that discussing TRD without first outlining a psychotherapeutic approach alongside pharmacotherapy is deemed untenable (Barsaglini et al., 2014; Steinert et al., 2014).

However, research on the effectiveness of psychotherapy in treating TRD is comparatively scarce compared to pharmacological approaches. Consequently, the relative efficacy of various

psychotherapeutic interventions remains largely unknown. Recent studies aimed at validating psychotherapy as an adjunct to pharmacotherapy have investigated cognitive behavioral therapy (CBT), the cognitive-behavioral analysis system of psychotherapy (CBASP), radically open dialectical behavioral therapy (RO-DBT), and behavioral activation (BA).

Meta-analytic investigations focusing on CBT have reported effect sizes (Cohen's *d*) ranging from 0.23 to 0.38 in depression severity scores compared to control groups (Cuijpers et al., 2010; Wiles et al., 2016). However, most participants completed at least five sessions of the same intervention before enrolling in these studies, potentially biasing results towards those still benefiting from therapy. Moreover, these studies typically included subjects with TRD level 1 (one failed antidepressant trial), potentially limiting their applicability to individuals with more severe resistance.

For CBASP, a meta-analysis revealed effect sizes (Cohen's *d*) varying from 0.34 to 0.50 in depression severity, with remission rates ranging between 19% and 57% in experimental groups compared to 6% to 50% in control groups (Michalak et al., 2015; Negt et al., 2016). Critics argue that CBASP was primarily developed for dysthymia treatment and may not be parsimonious, requiring a minimum of 30 sessions.

Randomized trials of RO-DBT reported post-treatment remission rates of 71% in the experimental group compared to 47% in the control group, with corresponding rates of 75% and 31% after six months (Lynch et al., 2007; Lynch et al., 2003; Lynch et al., 2015; Lynch et al., 2018). Notably, RO-DBT was initially developed to treat internalizing personality disorders, suggesting its potential efficacy for TRD with comorbid personality disorders. However, its implementation is resource-intensive, requiring significant time and highly trained clinicians.

Behavioral Activation for Depression

The foundation of BA stems from a longstanding tradition of behavioral theory and research. BA is a brief, structured treatment that focuses on activating individuals by prompting behaviors facilitating contact with diverse and stable sources of positive reinforcement, with activity scheduling serving as its central strategy (Santos et al., 2021). Additionally, BA aims to reduce avoidance and escape behaviors (Martell et al., 2013) while fostering the development of coping skills (Kanter et al., 2009) to initiate change (Kanter & Puspitasari, 2012). As a clinical procedure, BA encompasses four competencies: presentation of behavioral activation, activity evaluation, activity scheduling, and addressing avoidance (Kanter et al., 2009; Puspitasari et al., 2013).

Studies evaluating BA for TRD report effect sizes (Cohen's d) ranging from 0.52 to 0.67 concerning waiting list (pharmacotherapy alone) comparisons, depression severity, life quality, and attachment to pharmacological treatment (Shinohara et al., 2013; Dobson et al., 2010). While no specific clinical trials exist for TRD, a comprehensive analysis (Coffman et al., 2007) within a broader randomized clinical trial compared the efficacy of BA, CBT, and a Selective Serotonin Reuptake Inhibitor (SSRI) (Dimidjian et al., 2006). Results indicated that individuals with severe, longstanding interpersonal problems and depression responded better to BA (plus an SSRI) compared to CBT.

BA is recognized as a therapeutic intervention with robust evidence for MDD treatment by Division 12 of the American Psychological Association (APA), supported by at least two well-designed studies conducted by independent researchers (APA, 2022). Single-case experimental designs (Barraca, 2010) and nomothetic investigations have contributed to meta-analytic evidence demonstrating BA's efficacy compared to various pharmacological and psychological treatments, particularly CBT, with favorable outcomes for BA (Cuijpers et al., 2010).

Moreover, BA has demonstrated utility in environments where traditional clinical treatments are challenging due to time constraints or limited human resources, such as public health contexts. Another advantage of BA over other therapies is its relatively shorter training period for therapists and

implementation length, as it addresses specific issues and promotes recovery in a reduced number of sessions.

Coherently with the above revision, the aim of this paper is to propose a theoretical therapeutic plan for TRD and outline an intervention protocol for its implementation.

Behavioral theory of Treatment-resistant Depression

Similar to the behavioral theory of depression in BA the behavioral theory of TRD (BT-TRD) posits depression as a reinforcement trap (Baum, 2017). In this trap, individuals adhere to pharmacological and psychotherapeutic treatments that have demonstrated effectiveness in order to avoid delayed gratification, immediate distress, or the low probability of gratification, thus perpetuating the cycle of depression.

According to the theory, the relief derived from these strategies involves negative reinforcement of avoidance and escape behaviors, leading to an increase in the frequency and duration of negative behaviors. Over time, these response patterns can become ingrained, evolving into a lifestyle characterized by avoidance. Consequently, there is a progressive decrease in the frequency, persistence, duration, and complexity of pleasant activities or problem-solving efforts, leading to further dissatisfaction and the generation of additional stressors. These stressors are coped with using the same avoidant strategies, perpetuating an endless repetitive cycle.

The theory suggests that, over time, the activity pattern directed towards distress avoidance may lose its efficacy or generate intermittent positive reinforcement. This unpredictability resembles the variable reinforcement programs associated with persistence, even if contingent reinforcement does not consistently follow the behavior (Ferster & Skinner, 1957).

Types of adversities and consequences in BT-TRD

According to Ferster's Theory of Depression (1973), depressive states arise from individuals encountering adversity and its associated effects, resulting in a sudden decrease in positively reinforced activities. This decrease can stem from various sources, such as life changes leading to fewer opportunities for engaging in pleasant activities, the devaluation of rewards associated with these activities, or an increase in avoidance and escape behaviors competing with the pursuit of pleasurable activities. Lewinsohn (1974) described a similar condition, noting negative moods, reduced engagement in pleasant activities, and the emergence of avoidance and escape behaviors due to a decrease in contingent positive reinforcement.

Another adverse scenario involves a sudden influx of unwanted effects in an individual's usual activities or an increase in stressors unrelated to their actions. These factors can lead to negative moods, heightened avoidance behaviors, and the development of despair cognitions and low self-efficacy, also known as learned helplessness (Overmier & Seligman, 1967). Poor coping skills in the face of new stressors can exacerbate this condition, resulting from the failure of coping attempts and the eventual establishment of avoidance and escape patterns.

Additionally, some individuals may struggle to organize and regulate their behavior effectively, leading to persistent but ineffective coping strategies. In certain cases, individuals may rigidly adhere to rules such as "I must feel good before changing my habits or solving my problems", perpetuating avoidance behaviors. Instead of improving their situation, these avoidant mechanisms can exacerbate their circumstances, leading to further losses and stressors and reinforcing avoidance and escape behavior patterns (Martell et al., 2001).

In summary, (a) life changes understood as a decrease in contingent reinforcement of regular activities, the loss of opportunities to engage in pleasant activities, the sudden increase in stressors or unwanted effects contingent to the individual's habits, and possible deficits in coping skills when faced with adversity would result in (b) a decrease in behaviors oriented to positive reinforcement and an

increase in avoidance and escape behaviors from the stressful or deprivation experience generated by the conditions previously described. (c) Both, the reinforcement of this avoidance and escape behaviors, and a possible rule following of the type “I must feel good before I can change my habits or solve my problems”, and possible difficulties to organize behaviors in persistent activity patterns in the face of adversity, result in, (d) the establishing of a lifestyle oriented to the avoidance of psychological distress that results in more reward loss and the surging of bigger stressors, in an abulic and anxious mood, in hopelessness cognitions, an external locus of control, low self-efficacy, low self-esteem, and hypoactivity in reward circuits, characteristics of depression. (e) Eventually, intermittent reinforcement of avoidance and escape behaviors (and their eventual positive reinforcement), the use of medication or psychotherapy with the same avoidant goals, and the deterioration (due to disuse) of coping skills oriented to reward, problem-solving, and subsequent efficacy feeling included in pleasant activity patterns, would generate a chronic depressive cycle resistant to treatment. Theoretically, dispositional factors such as disorders in the secondary neuroendocrine functional systems, non-genetic variants, weak support systems, early-onset depressive cycles with the presence of other affective symptoms, pre-existing interpersonal behavior disorders, or substance abuse, are associated with the persistence of these depressive loops and its resistance to otherwise effective depression treatments. See Figure 1 for a synthetic representation of this theory.

— FIGURE 1 ABOUT HERE —

Intervention Protocol BA-TRD

The BA-TRD intervention protocol consists of thirteen sessions aimed at generating personalized routines or activity patterns to enhance reward efficacy and enrichment across various life domains, including social relationships. After initially assessing whether the therapy is suitable for the case and

collecting the necessary data to complete the functional analysis; in the intervention phase and in accordance with the BT-TRD, the primary objective of the program is to decrease avoidance and escape behaviors that impede problem-solving, compliance with self-care habits, and fulfillment of social obligations necessary for proper social adjustment. Secondly, the protocol focuses on the establishment of appropriate habits, their reinforcement, and their generalization and maintenance to prevent relapses. See Figure 2 for a description of the phases and sessions of the BA-TRD intervention protocol.

————— FIGURE 2 ABOUT HERE —————

Discussion

Given its relevance and severe impact on health systems, TRD should be considered a problem to be addressed by psychological therapies, just as it has been in pharmacological ones. With this approach, and based on the BT-RDT, the BA-TRD protocol was developed to address alternative interventions for TRD. This protocol can be applied as a standalone treatment or in combination with pharmacological approaches. The intervention has the primary advantage of being brief, and it is estimated that its training for psychologists who are familiar with the BA model would require no more than up to eight hours of training. It is manualized and clearly defines the behavioral (positive reinforcement) mechanisms within the framework of BA. Future studies, which are already underway, will specify the training program and assess the impacts of the protocol described in this article.

Although it is true that this protocol still requires empirical work for validation in TRD, it should be emphasized that it is based on the same principles of interventions with well-proven evidence in cases with severe MDD (Coffman et al., 2007). In addition, BA has also shown in large samples a better cost-effectiveness compared to antidepressant medication (Ekers et al., 2014) or with

CBT (Muzzecchelli et al., 2010), including also large public health systems, such as the UK's (Richards et al., 2016). Finally, we can say that the protocol adheres to criteria for maximum quality in experimental contrasting, emphasizing parsimony and ease of implementation (Cougale, 2012).

References

- Álvarez, E., Baca, E., Bousoño, M., Eguiluz, I., Martín, M., Roca, M. & Urretavizcaya, M. (2008).
Depresiones resistentes. *Actas Españolas de Psiquiatría*, 36(2), 35-43.
- APA, Asociación Americana de Psicología (24 marzo 2022). *División 12. Activación Conductual para la Depresión*. <https://div12.org/treatment/behavioral-activation-for-depression/>
- Barraca, J. & Pérez-Álvarez, M. (2015). *Activación conductual para el tratamiento de la depresión*. Síntesis.
- Barraca, J. (2010). Aplicación de la Activación Conductual en un Paciente con Sintomatología Depresiva. *Clínica y Salud*, 21(2), 183-197. <https://doi.org/10.5093/cl2010v21n2a7>
- Barsaglini, A., Sartori, G., Benetti, S., Pettersons-Yeo, W., & Mechelli, A. (2014). The effects of psychotherapy on brain function: a systematic and critical review. *Progress in Neurobiology*, 114, 1-14. <https://doi.org/10.1016/j.pneurobio.2013.10.006>
- Baum, W. M. (2017). *Understanding behaviorism: Behavior, Culture, and Evolution (3rd edition)*. Wiley-Blackwell.
- Berlim, M., & Turecki, G. (2010). Definition, assessment and staging of treatment-resistant refractory major depression: a review of current concepts and methods. *Canadian Journal of Psychiatry*, 52(1), 46-54.

- Coffman, S. J., Martell, C. R., Dimidjian, S., Gallop, R., & Hollon, S. D. (2007). Extreme nonresponse in cognitive therapy: can behavioral activation succeed where cognitive therapy fails? *Journal of Consulting in Clinical Psychology*, 75(4), 531-541. <https://doi.org/10.1037/0022-006X.75.4.531>
- Corrales, A., et al. (2020). Consenso argentino sobre el diagnóstico y tratamiento del Trastorno depresivo mayor resistente al tratamiento (TRD). *VERTEX Revista Argentina de Psiquiatría*, XXXI, 1-40.
- Cougle, J. R. (2012). What makes a quality therapy? A consideration of parsimony, ease, and efficiency. *Behavior Therapy*, 43(3), 468-481. <https://doi.org/10.1016/j.beth.2010.12.007>
- Cuijpers, P., Van Straten, A., Schuurmans, J., Van Oppen, P., Hollon, S. D., & Andersson, G. (2010). Psychotherapy for chronic major depression and dysthymia: a meta-analysis. *Clinical Psychology Review*, 30, 51-62.
- Dimidjian, S., Hollon, S. D., Dobson, K. S., Schmaling, K. B., Kohlenberg, R. J., Addis, M. E., Gallop, R., McGlinchey, J. B., Markley, D. K., Gollan, J. K., Atkins, D. C., Dunner, D. L., & Jacobson, N. S. (2006). Randomized trial of behavioral activation, cognitive therapy, and antidepressant medication in the acute treatment of adults with major depression. *Journal of Consulting in Clinical Psychology*, 74(4), 658-670. <https://doi.org/10.1037/0022-006X.74.4.658>
- Dobson, K. S., Hollon, S. D., Dimidjian, S., Schmaling, K. B., et al. (2010). Randomized trial of behavioral activation, cognitive therapy and antidepressant medication in the prevention of relapse and recurrence in major depression. *Journal of Consulting and Clinical Psychology*, 76(3), 468-477. <https://doi.org/10.1037/0022-006X.76.3.468>
- Ekers, D., Webster, L., Van Straten, A., Cuijpers, P., Richards, D., & Gilbody, S. (2014). Behavioural activation for depression; an update of meta-analysis of effectiveness and sub group analysis. *PLoS One*, 9(6), e100100. <https://doi.org/10.1371/journal.pone.0100100>
- Fawcett, J., & Kravitz, H. M. (1985). The long-term management of bipolar disorders with lithium, carbamazepine, and antidepressants. *The Journal of clinical psychiatry*, 46(2), 58-60.

- Ferster, C. B. (1973). A functional analysis of depression. *American Psychologist*, 28,857-870.
<https://doi.org/10.1037/h0035605>
- Ferster, C. B. & Sinner, B.F. (1957). *Schedules of reinforcement*. Appleton-Century-Crofts.
- Heimann, H. (1974). Therapy-Resistant Depressions: Symptoms and Syndromes: Contributions to Symptomatology and Syndromes. *Pharmacopsychiatry*, 7(3), 139-144. <https://doi.org/10.1055/s-0028-1094412>
- Kanter, J., Busch, A., & Rusch, L. (2009). *Behavioral Activation: distinctive features*. Routledge.
- Kanter, J., & Puspitasari, A. J. (2012). Behavioral Activation. In W. O'Donohue & J. E. Fischer (Eds.), *Cognitive Behavior Therapy: Core Principles for Practice* (pp. 215-250). John Wiley & Sons.
<https://doi.org/10.1002/9781118470886.ch9>
- Lewinsohn, P. M. (1974). A behavioral approach to depression. In R. J. Friedman & M. M. Katz (Eds.), *Psychology of depression: Contemporary theory and research* (pp. 157-178). Wiley.
<https://doi.org/10.4236/jcc.2015.310003>
- Lynch, T. R., Cheavens, J. S., Cukrowicz, K. C., Thorp, S. R., Bronner, L., & Beyer, J. (2007). Treatment of older adults with co-morbid personality disorder and depression: a dialectical behavior therapy approach. *International Journal of Geriatric Psychiatry*, 22, 131-143.
<https://doi.org/10.1002/gps.1703>
- Lynch, T. R., Morse, J. Q., Mendelson, T., & Robins, C. J. (2003). Dialectical behavior therapy for depressed older adults: A randomized pilot study. *American Journal of Geriatric Psychiatry*, 11, 33-45. <https://doi.org/10.1097/00019442-200301000-00006>
- Lynch, T. R., Whalley, B., Hempel, R. J., et al. (2015). Refractory depression: mechanisms and evaluation of radically open dialectical behaviour therapy (RO-DBT) [REFRAMED]: protocol for randomised trial. *BMFJ Open*, 5. <https://doi.org/10.1136/bmjopen-2015-008857>

- Lynch, T. R., Hempel, R. J., Whalley, B., Byford, S., Chamba, R., Clarke, P., Clarke, S., Kingdon, D., et al. (2018). Radically open dialectical behaviour therapy for refractory depression. *Efficacy and Mechanism Evaluation*, 5(7). <https://doi.org/10.3310/eme05070>
- Martell, C. R., Addis, M. E., & Jacobson, N.S. (2001). *Depression in context: Strategies for guided action*. Norton.
- Martell, C.R., Dimidjian, S., & Herman-Dunn, R. (2013) *Activación conductual para la depresión: Una guía clínica*. Descleé De Brouwer.
- Mazzucchelli, T. G., Kane, R. T., & Rees, C. S. (2010). Behavioral activation interventions for well-being: A meta-analysis. *The Journal of Positive Psychology*, 5(2), 105-121. <https://doi.org/10.1080/17439760903569154>
- Michalak, J., Schultze, M., Heidenreich, T., & Schramm, E. (2015). A randomized controlled trial on the efficacy of mindfulness-based cognitive therapy and a group version of cognitive behavioral analysis system of psychotherapy for chronically depressed patients. *Journal of Consulting and Clinical Psychology*, 83, 951–63. <https://doi.org/10.1037/ccp0000042>
- Negt, P., Brakemeier, E. L., Michalak, J., Winter, L., Bleich, S., & Kahl, K. G. (2016). The treatment of chronic depression with cognitive behavioral analysis system of psychotherapy: a systematic review and meta-analysis of randomized-controlled clinical trials. *Brain and Behavior*, 6(8). <https://doi.org/10.1002/brb3.486>
- Overmier, J. B., & Seligman, M. E. P. (1967). Effects of inescapable shock upon subsequent avoidance and escape responding. *Journal of Comparative and Physiological Psychology*, 63(1), 28–33. <https://doi.org/10.1037/h0024166>
- Puspitasari, A., Kanter, J. W., Murphy, J., Crowe, A., & Koerner, K. (2013). Developing an online, modular, active learning training program for behavioral activation. *Psychotherapy (Chic)*, 50(2), 256-65. <https://doi.org/10.1037/a0030058>

- Reyes-Ortega, M. A., Strosahl, K. D., & Arroyo-Jiménez, C. (2019). Aspectos fundamentales de la evaluación y conceptualización de caso. In M.A. Reyes-Ortega & K.D. Strosahl: *Guía clínica de evaluación y tratamiento del comportamiento suicida* (pp. 79-110). Manual Moderno.
- Richards, D. A., Ekers, D., McMillan, D., Taylor, R. S., Byford, S., Warren, F. C., Barrett, B., Farrand, P. A., Gilbody, S., Kuyken, W., O'Mahen, H., Watkins, E. R., Wright, K. A., Hollon, S. D., Reed, N., Rhodes, S., Fletcher, E., & Finning, K. (2016). Cost and Outcome of Behavioural Activation versus Cognitive Behavioural Therapy for Depression (COBRA): a randomised, controlled, non-inferiority trial. *Lancet*, 388(10047), 871-880. [https://doi.org/10.1016/S0140-6736\(16\)31140-0](https://doi.org/10.1016/S0140-6736(16)31140-0)
- Santos, M. M., Puspitasari, A. J., Nagy, G. A., & Kanter, J. W. (2021). Behavioral activation. In A. Wenzel (Ed.), *Handbook of cognitive behavioral therapy: Overview and approaches* (pp. 235–273). American Psychological Association. <https://doi.org/10.1037/0000218-009>
- Shinohara, K., Honyashiki, M., Imai, H., Hunot, V., Caldwell, D. M., Davies, P., et al. (2013). Behavioural therapies versus other psychological therapies for depression. *Cochrane Database of Systematic Reviews*. <https://doi.org/10.1002/14651858.CD008696.pub2>
- Spijker, J., Van Straen, A., Bockting, C., Meeuwissen, J., & Van Balkom, A. (2013). Psychotherapy, antidepressants and their combination for chronic major depressive disorder: A systematic review. *Canadian Journal of Psychiatry*, 58(7), 386-392.
- Steinert, C., Hofman, M., Kruse, J., & Leichsenring, F. (2014). Relapse rates after psychotherapy for depression-stable long-term effects? A meta-analysis. *Journal of Affective Disorders*, 168, 107-118. <https://doi.org/10.1016/j.jad.2014.06.043>
- Wiles, N. J., Thomas, L., Turner, N., Garfield, K., Kounali, D., Campbell, J., et al. (2016). Long-term effectiveness and cost-effectiveness of cognitive behavioural therapy as an adjunct to pharmacotherapy for treatment-resistant depression in primary care: follow-up of the CoBaIT

randomised controlled trial. *Lancet Psychiatry*, 3(15), 137–44. [https://doi.org/10.1016/S2215-0366\(15\)00495-2](https://doi.org/10.1016/S2215-0366(15)00495-2)

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Highlights

- Treatment-Resistant Depression (TRD) is a complex condition that poses a crucial challenge for clinicians and public health. To date, approaches to this problem have been almost exclusively pharmacological, to the point that the definition of TRD is associated with failure with several psychopharmacological interventions and there is no consensus on its delimitation from psychological interventions.
- Behavioral Activation (BA) has updated the analysis and interventions in depression. Thanks to its contributions and a re-reading of learning principles possibly associated with depression, an explanatory model has been developed to account for TRD.
- Based on the explanatory model of TRD, a protocol of six phases and thirteen sessions has been developed for the approach of TRD. This protocol, which is presented in this paper, can be easily trained by therapists who are familiar with BA. Although it still requires more empirical work, its solid learning foundations and its link with previously successful BA interventions bode well for its usefulness to clinicians.

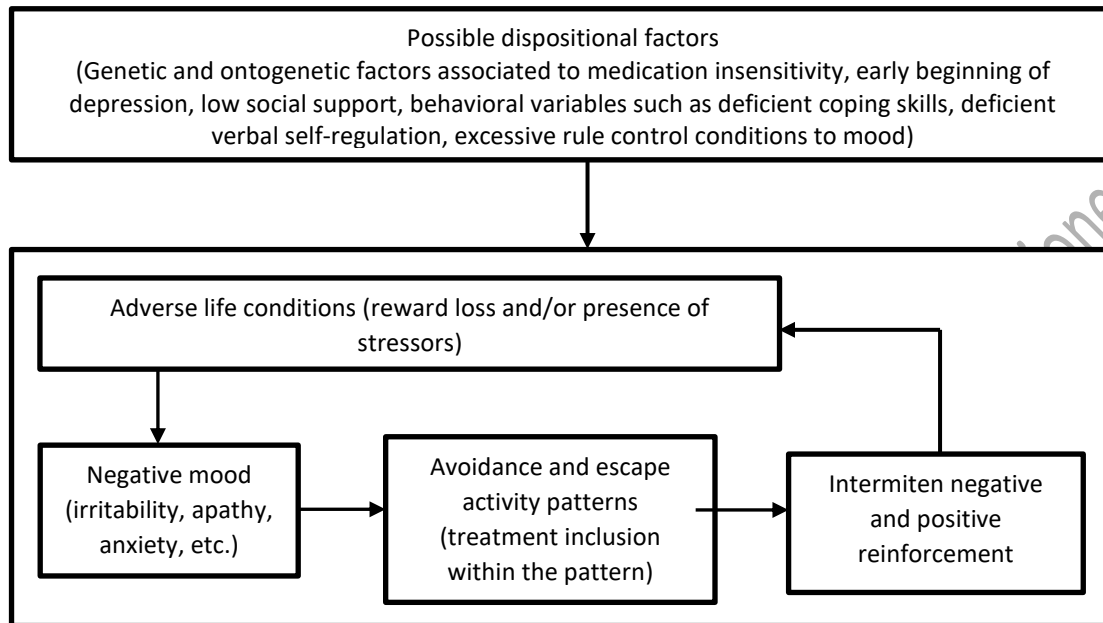
Figure 1.**Behavioral Theory of Treatment-resistant Depression (BT-TRD)**

Figure 2.**The General Structure of the Intervention Protocol BA-TRD***

Phase 1 Preparation for BA-TRD Aimed to evaluate the adequacy of BA-TRD for the patient condition and to provide orientation about the program.	SESSION 0 – 60 minutes Session 0 begins obtaining sociodemographic and clinical identification data from the patient; then, through a simplified functional analysis the clinician presents TRD as a pattern of interaction with the environment characterized by behavioral avoidance and escape and BA as an alternative interaction pattern based on activity scheduling that creates a positive loop that promotes and maintain a contented and resilient lifestyle. If the patient understands and accepts the information, the therapist may obtain the patient's informed consent to take part in BA-TRD and apply clinimetry (it is suggested to apply BA, depressive symptomatology, quality of life, and general functioning measures).
Phase 2 Functional Analysis of Depression Phase 2 aims to develop an individualized case conceptualization and train the patient to functionally analyze their current depressive interactive pattern.	SESSION 1 – 60 minutes The clinician orients the patient about the session schedule and conducts a focused interview (Reyes-Ortega, Strosahl & Arroyo-Jiménez, 2019), in which the patient identifies challenges in interpersonal, academic, work, leisure, and health-related areas of their life. After, the patient uses a label to identify his/her experience, recall their first awareness of its appearance, course and the current circumstances that may worsen the pain, finally the patient identify attempted solutions to their depression. The clinician uses this information to present a functional diagram of the patient depression synthesizing negative life events, adverse experiences, and behavioral changes that may generate a negative loop (Kanter, Busch & Rusch, 2009). After the consultant understands and accepts such conceptualizations, they can receive a copy of this outline for them to agree to perform a functional analysis of depressive situations, reactions, and actions throughout their week.
	SESSION 2 – 60 minutes The session begins with a summary of the previous session and a review the functional analysis performed by the patient and the learned results. Later, the therapist and patient collaboratively describe the possible interaction between alternative activities, life circumstances, and the resulting affective events. Also, the patient receives and reviews a copy of The Ten Principles of Behavioral Activation by Martell et al. (2013). After understanding and discussing the principles, the patient may agree to perform a new functional analysis of depressive situations, reactions, and actions throughout the following week.
	SESSION 3 – 60 minutes The session begins with a summary of session 02, a review of the patient's functional analysis, and an exploration of the resulting learnings. The patient and therapist can then review and discuss the text Three Common Myths About Behavioral Activation and its Challenges by Kanter et al. 2009: That behavioral activation is an exercise in willpower, overt behavioral change depends on an initial cognitive or emotional change, and that depression is a brain illness exclusively, where life circumstances and behavior play no role. Then, the patient fills an activity monitoring format based on the works of Martell et al. (2013); it is explained that its function is to identify how current activities are related to their moods and identify opportunities to perform new activities. The format is filled in with the patient to train him or her in its completion, and after exploring generated learnings, subjects can make a copy with information obtained during the week. The patient is free to fill in the format in one or two moments throughout the day and report activities and their effect on the mood each hour.
Phase 3 Activity Scheduling Phase 3 intends to enhance understanding of the effect activities has on moods and, through daily monitoring, identifying, and prioritizing alternate activities to schedule. Also, the patient may begin performing scheduled activities based on the hierarchy given.	SESSION 4 – 60 minutes Session 04 begins by reviewing the patient's activity monitoring format and reviewing the learnings generated by the activity, emphasizing the relevance of monitoring to identify antidepressant activities. The clinician explains that the objective of filling the format is to evaluate activities that are part of an action plan that interrupts the maintaining cycle of depression and then to be able to improve quality of life. The therapist and client discuss the ways in which following an activation plan could have on life circumstances and resulting emotions. The clinicians explain that for a successful recovery, the patient should understand that they must explore multiple sources that help generate a varied, concrete, and focused plan of activities that can start immediately. The subject receives a worksheet with categories of activities that may be used as a basis for identifying possible behavioral activation activities: abandoned essential self-care habits, pleasing and self-efficacy feeling generating activities that have ceased, or that the patient may want to perform but not attempted, activities that must be done but are postponed or avoided (including steps to solve problems), and activities that are related to the accomplishment of goals and the feeling of more value in life. The patient rates them from most to least important and discusses two

	or three alternative activities in each given area. Given that it is challenging to finish generating all these alternatives, the consultant and therapist agree to fill in the format and continue filling in the self-monitor homework.
	SESSION 5 – 60 minutes Session 05 begins with reviewing the agreed strengthening activities and generated learnings. The patient and clinician discuss the information recorded in the activity scheduling format and identify three actions that the subject can schedule for the following week. These actions must belong to three different categories. This information aims to monitor and select two activities the patient commits to perform. The actions must be challenging enough that the patient feels gratified achieving them while still realistic enough for them to accomplish them. The patient will register these actions in a format that contains the activity, the day, time, and place to perform them, people who might aid in their realization of the task, an estimation from 0-100% of activity completion, the attention given to the task, the effect on the subject after realization, and a final analysis of what the patient learned.
Phase 4 Dealing with avoidance and punishment This phase aims to schedule new activities and evaluate success in the performance and completion of these tasks and the rewards attained after successful activation.	SESSION 6-8 – 60 minutes Sessions 06 to 08 begin with the reviewing of the activity scheduling format. a) If a patient completed the agreed activities and perceived positive effects, he/she could choose to continue reinforcing these actions throughout the following week. If these activities represent a first step in pursuing a goal or problem solving, the patient can schedule the next step. The therapist can use the activity categories format to identify activities to schedule in other areas. The patient can also use a format stipulating rewards (Barraca-Mairal; Pérez-Álvarez, 2015) that they can choose to receive if they are successful with their activation plan. b) If the patient performed the agreed activities but did not experience positive effects, the therapist may substitute these tasks within the area. c) If the patient fails to complete any tasks, the therapist can use the TRAP-TRAC model described in Martell et al. (2013) to reinforce problem-solving skills. The recommendations for problem-solving (TRAC), Kanter et al. (2009) are the following: If the task is too challenging to perform, the subject may fragment the activity into smaller behavioral units. If the problem is forgetting the task, the patient may use stimulus control strategies. The subject may ask for positive social reinforcement or help from a close relative. If the patient's social network inhibits the completion of the task, they can implement brief mindfulness exercises to counter rumination. At the end of the session, the patient fills out a new activity scheduling format and agrees to perform these tasks.
Phase 5 Completion Phase 5 includes sessions 09 and 10, aiming to prepare the consultant for the completion of therapy and to prevent relapses.	SESSION 9 – 60 minutes Session 09 has the same structures as sessions in phase 4. The difference is in the presentation of a relapse prevention plan that the patient can use if he or she notices relapsing in the future. For this, the therapist may use a format based on the works of Martell et al. (2013). It asks the patient to identify events associated with mood worsening and then make a functional outline similar to the ones used in sessions 01 and 02. The patient then identifies actions that they can take to interrupt the depressive cycles and improve their mood. Finally, the consultant identifies strategies they can use to stay motivated throughout this plan.
	SESSION 10 – 60 minutes Session 10 has the same structure as session 09. However, instead of making a relapse prevention plan, the patient and therapist discuss using the acronym ACTION developed by Martell et al. (2010) to remember to maintain behavioral activation as a lifestyle. The consultant then schedules new activities and incorporates the logic of BA into their daily life. Finally, the consultant and therapist bid farewell momentarily and give each other feedback about any learned experience. Aspects of BA-TRD that seemed helpful are identified collaboratively and to celebrate their results.
Phase 6 Maintenance The main objective of this phase is to monitor relapse prevention maintenance plan	SESSION 11 – 60 minutes Session 11 occurs one month after session ten and lasts for 45 minutes, during which the patient receives orientation about basic sleep hygiene measures, problem behaviors associated with poor sleep are also identified and changes are agreed. Later, the patient and therapist review a relapse-prevention plan and seek tasks that reinforce the success in activation maintenance (if necessary) and analyze the TRAP-TRAC model for possible challenges presented and their solutions (if necessary).

and problem-solving associated with its implementation and orient about sleep hygiene measures and mindfulness practices.

SESSION 12 – 60 minutes

Session 12 occurs three months after session ten and has a duration of 30 minutes, during which the patient and therapist review the relapse prevention plan and the previously scheduled activities. The therapist and subject reinforce the success in the activity by maintaining and analyzing with the TRAP-TRAC model the challenges presented and possible solutions to perform. The consultant is invited to incorporate the logic of BA into his or her daily life, and finally, the consultant and therapist bid farewell and close the therapeutic process.

* Please contact the corresponding author to obtain the full protocol, including detailed descriptions of the activities performed in each session.

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SESSION 1 DESCRIPTION

Reyes-Ortega & Zapata-Téllez, 2021

SESSION: 1	PHASE: Initial Evaluation	DURATION: 60 min
OBJECTIVE:	General presentation, conduct an initial interview, perform a functional analysis of the current depressive episode.	
MATERIALS:	Ψ Format 2: Initial interview for the clinician Ψ Format 3: My own functional analysis Ψ Format 4: My own functional analysis - Homework	
ACTIVITY SCHEDULING:		
1. General presentation → <i>Format 2</i> a. Personal presentation b. Present the session’s agenda c. Orientation on the dynamics of the session		5 min
2. Conducting an initial interview → <i>Format 2</i> a. Explore life contexts by asking brief questions that will quickly help understand how the client’s life is. b. Identify affected life contexts (routine disruptions) and behavior patterns related to depression. c. Exploring the current episode of depression, assessing subjective severity, trajectory, duration, and turbulence. d. Presenting a summary of the main environmental components of the history and their emotional impact on the client, focusing on depression as a normal human reaction to difficult and negative life events.		30 min

3. Conducting a functional analysis of the current depressive episode. → <i>Format 3 (fill out together with the consultant)</i> a. Identify negative life experiences (Table 1). b. Identify the emotional reactions the client had to those experiences (Table 2). c. Identify how the client acts in response to those emotional experiences/avoidance patterns (Table 3)." d. Explain how these reactions and actions would be natural and frequent in response to these life events. e. Explain how these reactions and actions form a loop that perpetuates or exacerbates negative life circumstances and the experience of suffering.	15 min
4. Session summary, assign activities for the week, and session closing	10 min
ACTIVITIES FOR THE WEEK:	Conduct a functional analysis of depression in the current week → <i>Format 4 (to be provided to the client)</i> a. Ask the client to identify two negative life experiences that have occurred each day, as well as the emotional reactions to those events and their behavioral avoidance pattern (how the emotional reactions affected their behavior).

Format 2
Initial Interview for the
Clinician

Hello, my name is _____. I am a psychotherapist and my job is to help people develop healthy behaviors, and better quality of life.

Today I will ask you several questions in order to understand the general context of your life and the problems associated with depression that are troubling you. Then, I will spend a few more minutes familiarizing myself with the nature of the episode that is currently troubling you. And finally, we will spend the rest of the time developing an outline to help us look at how things have been going in your life, and what we can do during subsequent sessions to make things better.

This will be a session in which we will be gathering a lot of information and I may talk a lot more than you. To make the most of the time I will ask you to try to be very brief in your answers and allow me to lead the interview, okay, any questions?

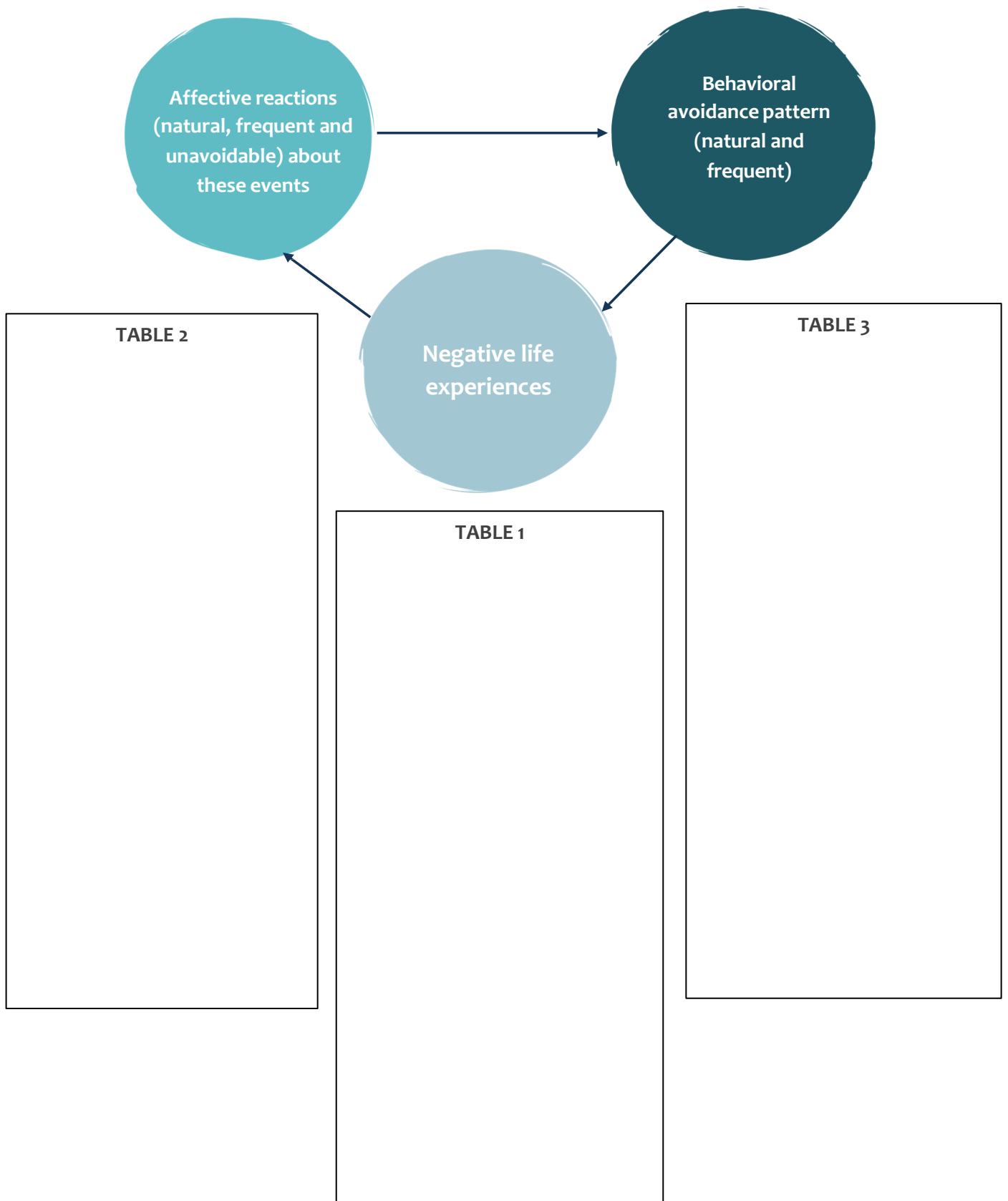
Relationships Who do you live with?, how does everyone get along?, who are the people you are the closest to?, family, friends, partner	
Occupation What do you do?, do you enjoy your occupation?, do you get along well with with your colleagues?	
Leisure What do you do for fun?, to relax, to connect with people or a community?	

Health Do you use legal and/or illegal drugs? Do you spend a lot of time on social networks? Do you exercise regularly? Do you eat and sleep well? Do you have any diseases?	
Current Problem Severity How severe do you consider your depression to be?	() Little disturbing () Very annoying () Severe. () Incapacitating Why?
Current Problem Time When did this problem start?	
Trajectory What seems to make your problem worse?, what seems to make your problem better?	
Current Problem Turbulence Is there an event or person that seems to instigate or aggravate the problem at this time?	
Current Problem Solutions What have you tried to do to solve or cope with this problem?	

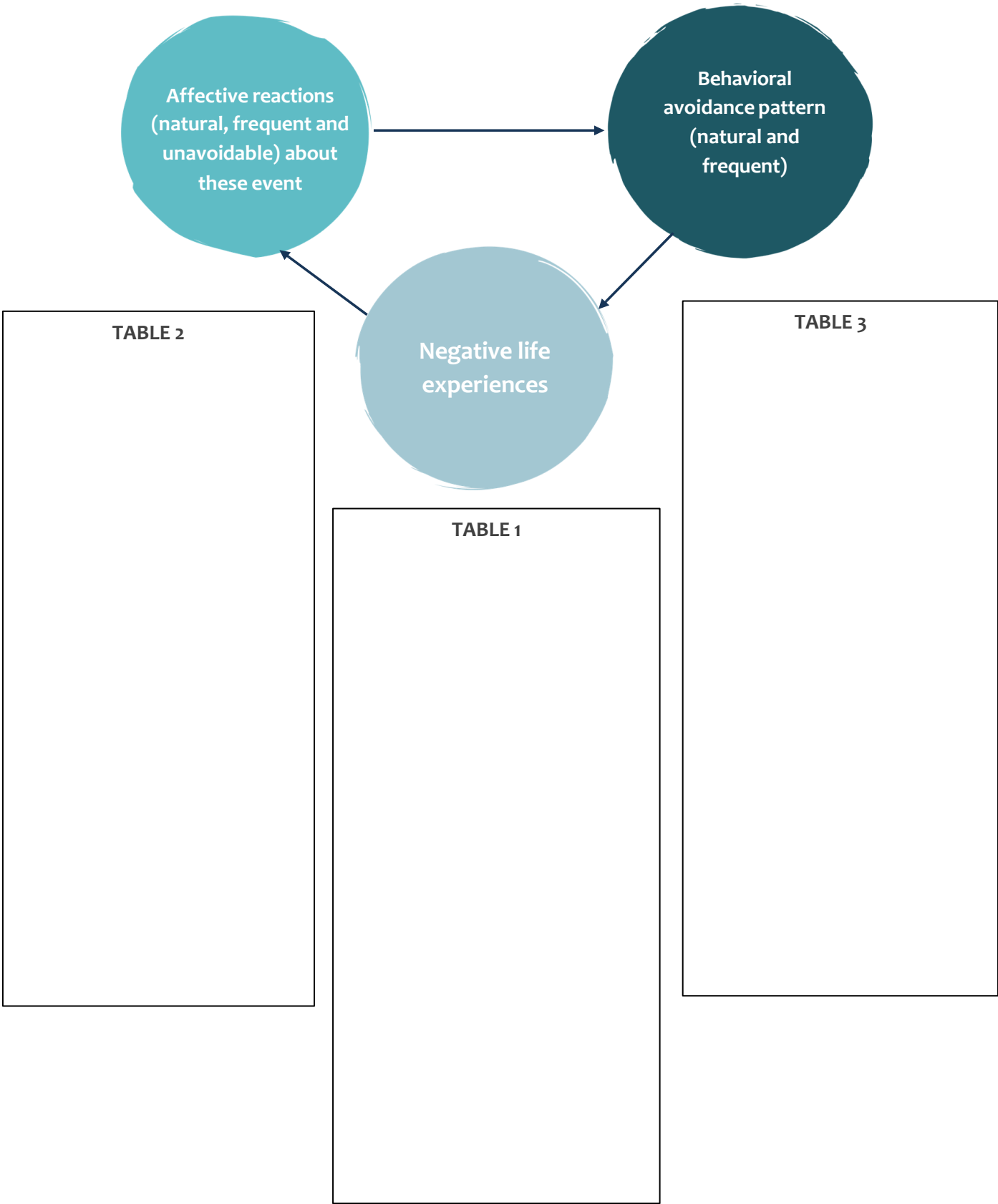
"Now I would like to show you the idea I have of how this depression is formed and maintained."

Format 4

My Own Functional Analysis



Format 5
My Own Functional Analysis – HOMEWORK



SESSION 2 DESCRIPTION

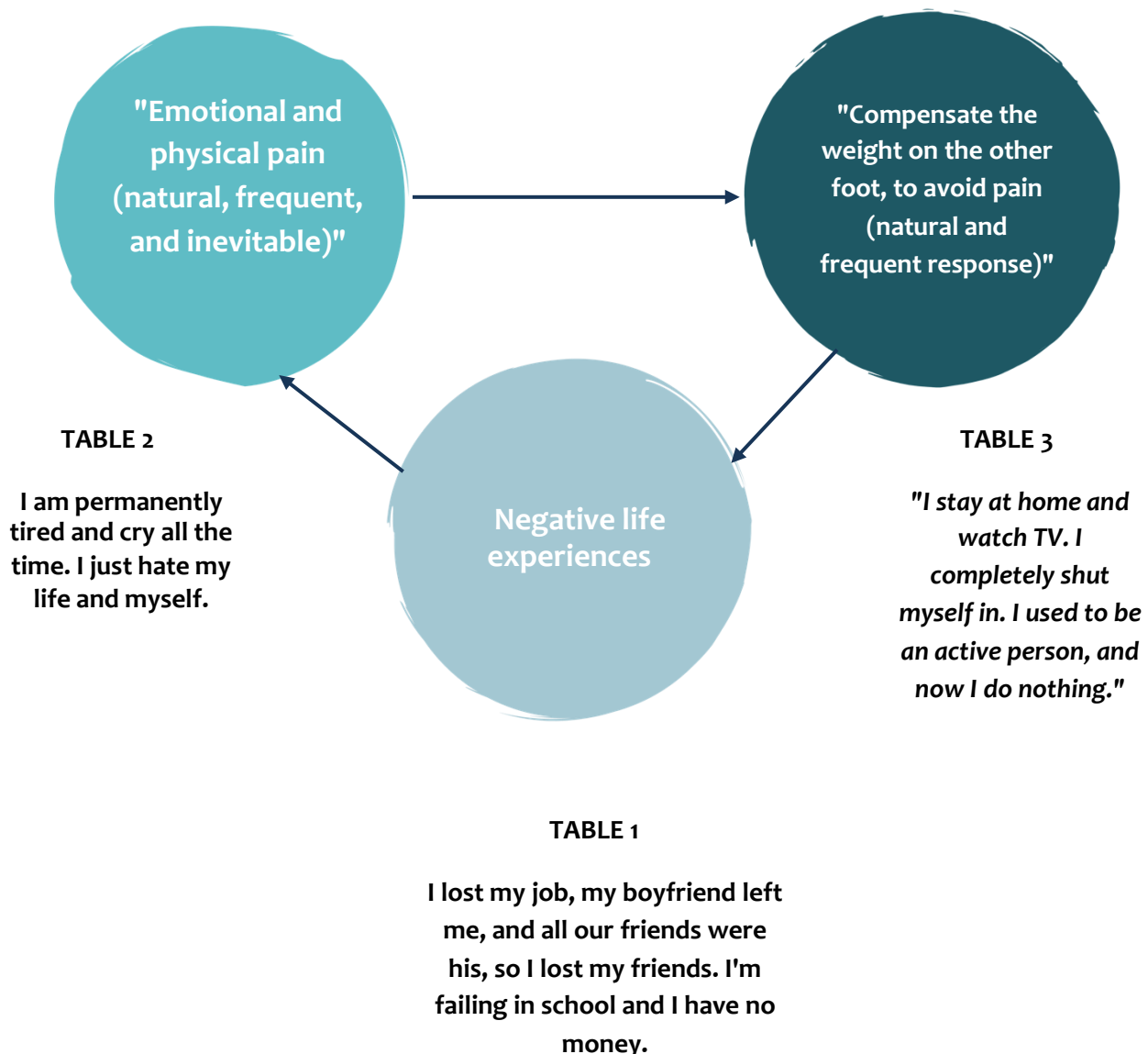
Reyes-Ortega & Zapata-Téllez, 2021

SESSION: 2	PHASE: INTRODUCTION TO BA	DURATION: 60 min
OBJECTIVE:	Review homework, present depression from a metaphorical perspective of the dislocated ankle, present the rationale and principles of Behavioral Activation, and introduce activity monitoring.	
MATERIALS:	Ψ <i>Format 5</i> : The dislocated ankle metaphor Ψ <i>Format 6</i> : Depression from a behavioral perspective Ψ <i>Format 7</i> : The Rationale for Behavioral Activation	
ACTIVITY SCHEDULING:		
1.	Present the summary of the previous session and the objectives of the current session.	5 min
2.	Review the weekly task by linking it to the dislocated ankle metaphor. → <i>Format 5</i>	15 min
3.	Presenting depression from a behavioral perspective → <i>Format 6</i> 3.1. Explain that there is an event (a situation or a set of situations) that results in a decrease in pleasant and stimulating activities, which may be accompanied by an increase in threatening, stressful or frustrating events/activities. Another possibility is that there is an increase in activities with adverse results, without necessarily a decrease in rewarding activities. 3.2. Explain that as a consequence of these conditions, people experience a lack of motivation (they do not experience an urge to do things because no incentives are available) and/or an increase in discomfort (they experience the urge to avoid because activities lead to the occurrence of adverse consequences). 3.3. Explain that the problem lies in the fact that the natural reaction in these cases is adopting a passive or avoidant style of action, which works very well in the short term to reduce, avoid or postpone adverse consequences and the discomfort associated with them. 3.4. Explain that, in the long run, passivity and avoidance lead to lower exposure to stimulating activities and, therefore, maintain the adverse conditions, resulting in a rigidity and generalization of this interaction pattern with the environment (or, in other words, of this painful lifestyle).	15 min

4. Present the rationale for Behavioral Activation. → Format 7 4.1. Explain that the aim of Behavioral Activation is to "activate" as an alternative to the normal avoidant behavioral responses to suffering. Find alternative actions that the consultant could take, no matter how fantastic they may seem to you (Table 1). 4.2. To help the consultant identify the alternative life effects that these alternative life actions could have (Table 2) 4.3. Helping the consultant to notice the effects that such life actions could have in their affective responses (Table 3). 4.4. Verify the understanding that the client has developed about how behavioral activation works.		20 min
5. Session summary and closing		5 min
ACTIVITIES FOR THE WEEK:	Reviewing depression from a behavioral perspective → Format 6	

Format 5
The Luxating Ankle Metaphor

An episode of depression is very similar to an episode of physical discomfort. Think of your negative life experiences such as dislocating an ankle, it is very natural to experience discomfort in such a situation, both physically and emotionally, and you may even notice a cascade of negative thoughts as a consequence. The normal thing to do, and what most people would do, would be to start putting our body weight on the other foot to avoid feeling pain. Inadvertently, that could lead us to injure the other ankle, increasing the degree of pain experienced, and furthermore, it would lead us to keep the injury of the first ankle unhealed.



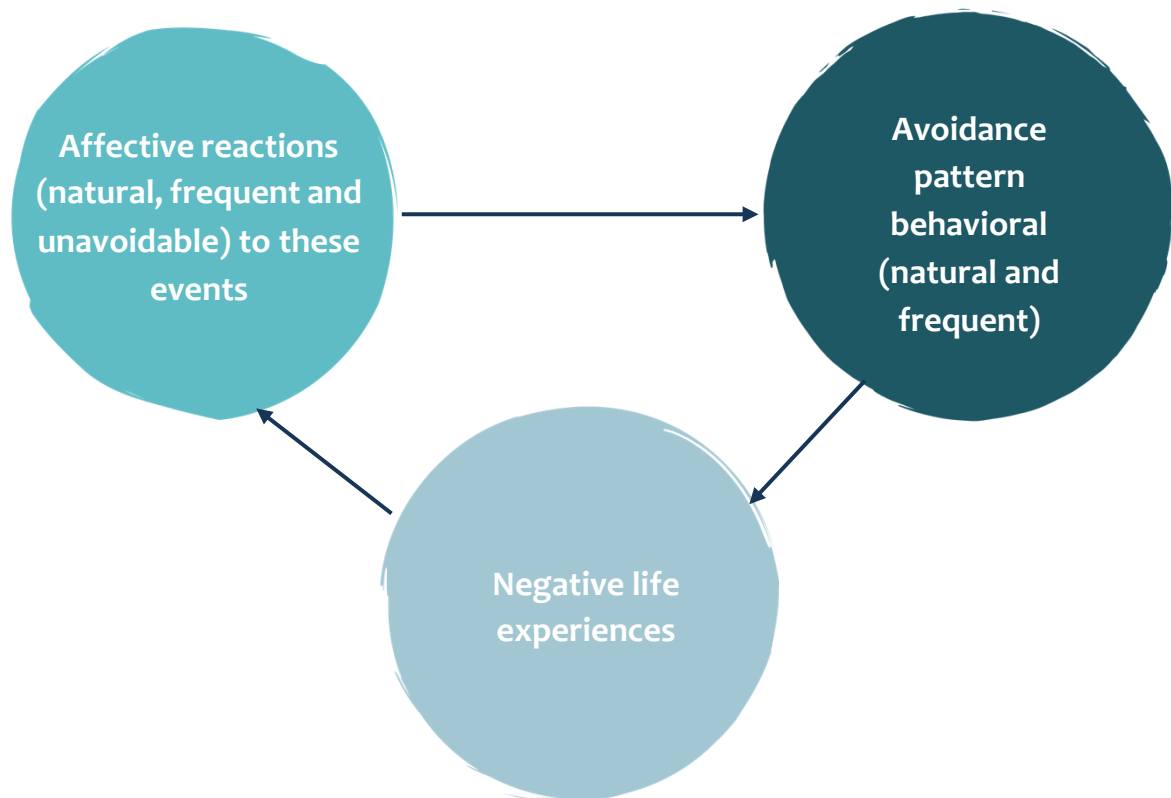
Do you find similarities or differences with your particular case?

Format 6

Depression From a Behavioral Perspective

Behavioral Activation understands depression as a pattern of interaction between the person and his or her environment:

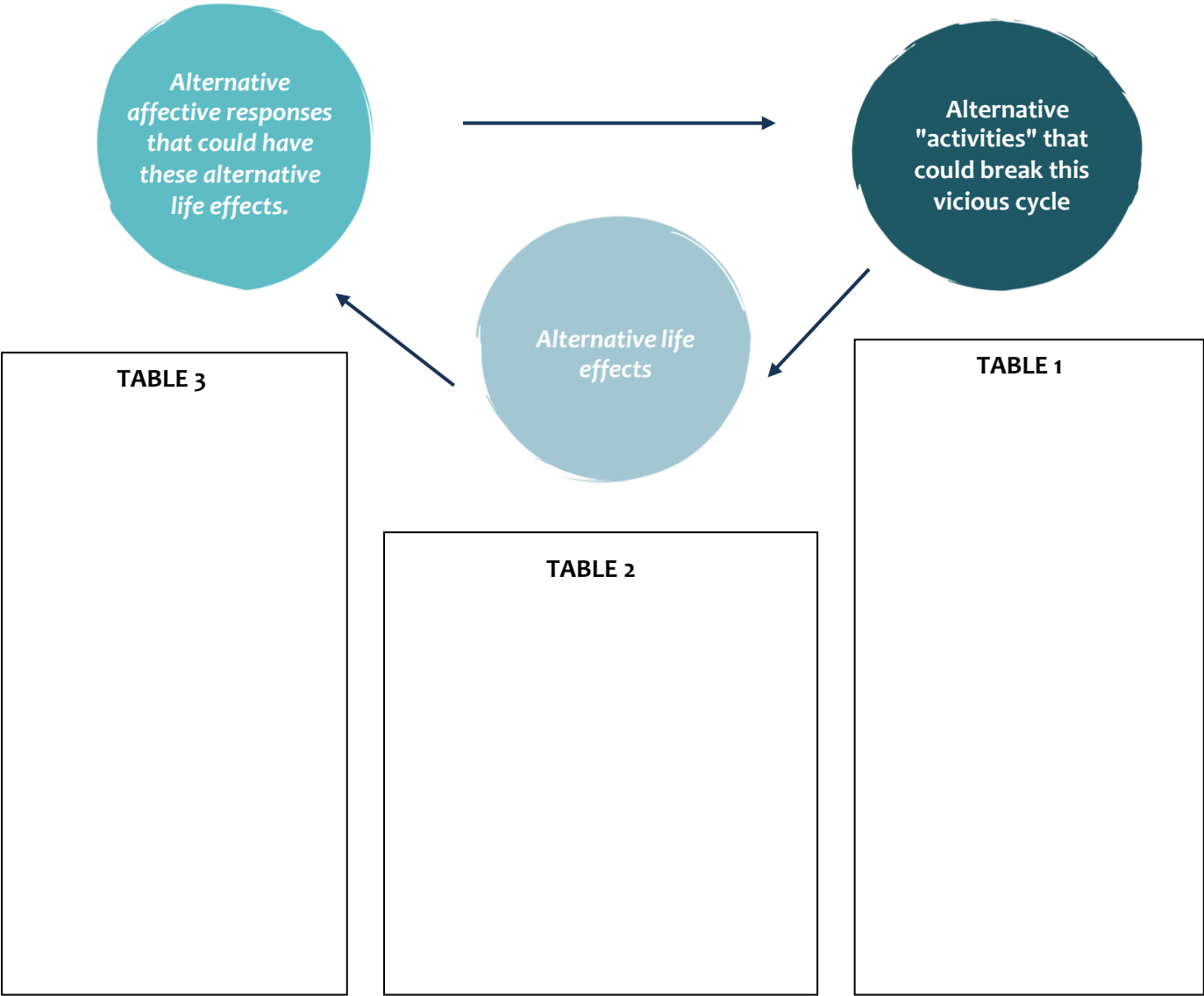
1. Firstly: There is a situation or set of situations that lead to a decrease in enjoyable and stimulating activities. This may or may not be accompanied by an increase in stressful or frustrating events/activities. Another possibility is that there is an increase in activities with adverse outcomes, without necessarily a decrease in rewarding activities.
2. As a consequence of these conditions, people experience a lack of motivation (they do not experience an urge to do things because no incentives are available) and/or an increase in discomfort (they experience the urge to avoid because activities lead to the occurrence of adverse consequences).
3. The problem is that the natural reaction in these cases is to adopt a passive or avoidant style of action, which works very well in the short term to reduce, avoid or postpone the adverse consequences and the discomfort associated with them.
4. But in the long run, passivity and avoidance lead to less exposure to stimulating activities and thus maintain adverse conditions, resulting in a rigidity and generalization of this pattern of interaction with the environment (or, in other words, this painful lifestyle).



Format 7
Rationale of Behavioral Activation

Behavioral Activation is like doing physical rehabilitation and starting to move your ankle to heal the injury. Discuss with your psychotherapist how you might do different activities that will lead you to break the spiral of suffering you are in, and heal that ankle once and for all.

- ✓ Write down the alternative actions you could take (no matter how fantastic they may seem to you) in Table 1
- ✓ Note the alternative life effects that these alternative life actions could have and write them down in Table 2
- ✓ Note the effects that these alternative life actions could have in your affective responses and write them down in Table 3.



SESSION 3 DESCRIPTION

Reyes-Ortega & Zapata-Téllez, 2021

SESSION: 3		PHASE: Activity Monitoring	DURATION: 60 min
OBJECTIVE: Present the principles and myths of Behavioral Activation, introduce the daily monitoring form.			
MATERIALS: Ψ Format 8: Rationale and Principles of Behavioral Activation Ψ Format 9: Myths of Behavioral Activation Ψ Format 10. Instructions. Daily monitoring form Ψ Format 11. Daily monitoring form			
ACTIVITY SCHEDULING:			
1. Present the summary of the previous session and the objectives of the current session			5 min
2. Present the rationale and principles of Behavioral Activation →Format 8 2.1. Point out that the logic of the BA is to change the behavior in order to generate improvements in thoughts, feelings and quality of life in general. 2.2. Point out that instead of working with the logic "from the inside out", you work "from the outside to the inside" 2.3. Read and explain the 10 principles of BA			20 min
3. Presenting the myths of Behavioral Activation →Format 9 3.1. The myth of "looking forward to it" 3.2. The logic of inner change 3.3. The biological logic			10 min
4. Introduce and teach the completion of the Daily Monitoring Form →Format 10 A. Allows you to be in touch with the activities and their effect B. Evaluate changes throughout the sessions C. Generate ideas for activation tasks D. Evaluate the level of enjoyment and importance (0-10) for each activity carried out, and general mood at the end of the day			20 min
5. Session summary and closing			5 min
ACTIVITIES FOR THE WEEK:	Ψ Complete the Daily Monitoring Form → Format 11 Monitor moods, level of enjoyment and importance		

Format 8

The Rationale and Principles of Behavioral Activation

The core of this treatment is **Behavioral Activation**, which consists of changing your behavior in order to generate improvements in your **thoughts, feelings and general quality of life**.

When people become depressed, they tend to think “when I feel better, I will do this or that activity”, that is, they believe that only when they are feeling well will they be able to perform the important or pleasant activities, or solve the problems, that they are not performing at that moment.

MOOD → ACTIVITIES

In this treatment, however, we will take the opposite approach: we will focus primarily on doing these important or enjoyable activities to make ourselves feel better.

ACTIVITIES → MOOD

The proposal then involves modifying your actions and activities, which will help you feel more energy and motivation, and experience a better overall quality of life. We are not going to ignore your thoughts and feelings, but rather focus on your actions, so that you will have more positive life experiences. We suggest you think of your mood as the result of your actions, not as the cause.

Have you recently had a thought along the lines of, “When I feel better I'm going to...”?

MAIN POINTS

- The focus of the treatment is to change your actions
- Your mood will improve as you change your actions

The ten fundamental principles of Behavioral Activation

PRINCIPLE 1

- The key to changing how people feel is to help them change what they do. This does not make them false or incongruent.

PRINCIPLE 2

- Life changes can lead to depression, and short-term coping strategies can interfere with long-term goals.

PRINCIPLE 3

- The clues to understanding the actions that will be antidepressant for a person are in the antecedents and consequences of his or her actions.

PRINCIPLE 4

- To get out of depression, you must structure and schedule activities that follow a plan, not a mood.

PRINCIPLE 5

- Change is easier when it starts small and adapts to people's pace.

PRINCIPLE 6

- Change is more effective if important activities are considered, which have an immediate antidepressant effect and cover different areas of life.

PRINCIPLE 7

- The therapist will act as a coach.

PRINCIPLE 8

- Insist on an empirical approach to problem solving and recognize that all results are useful.

PRINCIPLE 9

- Don't talk about doing, DO!

PRINCIPLE 10

- Working to solve the obstacles to activation

Format 9
The Myths of Behavioral Activation

The myth of having the will	MYTH: Behavioral Activation is all about "willpower". FACT: Activation is more complex than people think. Behavioral activation is a collaborative effort, where you and your behavioral consultant will develop plans that fit your pace and possibilities. Your behavioral consultant will be like a coach who will teach you how to use strategies to help you initiate and maintain activation. In behavioral activation, there is no assumption that "willpower" will get you to activate and overcome this depressive episode.
The rationale of internal change	MYTH: The rationale of "inside-out" change (feeling good first to act afterwards) is common and makes sense to assume, as we've all learned it somewhere. FACT: The power of behavioral activation lies in acting differently first, to feel differently afterwards. Additionally, this builds greater self-confidence and increased tolerance for the natural discomfort that comes with this effort.
Biological logic	MYTH: Depression, anxiety, and stress are problems of our brain and, therefore, we need to take medication to correct them. FACT: Research has shown that Behavioral Activation changes the chemistry of our body, much like physical exercise changes our muscle tissues. FACT: Combining behavioral activation with pharmacotherapy can be very helpful in most cases, much like maintaining a diet and nutritional supplementation when exercising. Exercising will yield results, but taking supplements without exercising will be of minimal help.

Format 10

Instructions. Daily Monitoring Form

INSTRUCTIONS

The first activity we will do is to record your daily activities. For this, we will use the Daily Monitoring Form to:

1. Identify how your daily activities affect your mood
2. Note your current level of activity, which we can compare with the level of activity you will have later on."
3. Begin to consider what changes you could introduce in your life.

HOW IS IT DONE?

1. Record for each hour of the day the activity you have done, even if it seems insignificant.
2. Once the activity has been recorded, score it according to the level of **ENJOYMENT** and the **IMPORTANCE** it has at that moment. For the *Enjoyment* score, consider how much you are enjoying it, how enjoyable the activity is for you at that moment, and rate it from 1 to 10. A score of 1 means that you have not enjoyed the activity at all, and a score of 10 means that you have enjoyed the activity very much. For the *Importance* score, consider how important it is to your life to be doing that activity, and score it from 1 to 10, where 1 means that you feel the activity is not important at all, and 10 means that the activity is very important to your life.
3. Finally, at the end of each day, before going to bed, rate your overall mood for the day from 1 to 10, with 1 being the worst mood and 10 being the best.

For example, a log fragment might look like this:

HOUR	ACTIVITY	ENJOY 1-10	IMPORTANCE 1-10
10-11	Breakfast with friends	5	3
15-16	Working	2	7

WHEN TO COMPLETE THE FORM?

You can complete the form:

- As you go through your day, hour by hour
- Logging every two to three hours
- Logging everything at the end of the day

It is not a good idea to miss more than one day to complete it, as it is easy to forget the details of the activities and how you felt. We will be working with daily monitoring forms every session, so be sure to complete it and bring it with you each time you attend.

Format 11
Daily Monitoring Form

Hour	Day:	D	I	Day:	D	I	Day:	D	I	Day:	D	I			
0-5															
5-6															
6-7															
7-8															
8-9															
9-10															
10-11															
11-12															
12-13															
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15-16															
16-17															
17-18															
18-19															
19-20															
20-21															
21-22															
22-23															
23-24															
Overall Mood:				Overall Mood:				Overall Mood:				Overall Mood:			

Format 11
Daily Monitoring Form

Hour	Day:	D	I	Day:	D	I	Day:	D	I	Day:	D	I			
0-5															
5-6															
6-7															
7-8															
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20-21															
21-22															
22-23															
23-24															
Overall Mood:				Overall Mood:				Overall Mood:				Overall Mood:			

DESCRIPTION SESSION 4

Reyes-Ortega & Zapata-Téllez, 2021

SESSION: 4		PHASE: Activities to schedule	DURATION: 60 min
OBJECTIVE:	Review daily monitoring form, identify activities to be scheduled, make hierarchy of activities to be scheduled		
MATERIALS:	Ψ <i>Format 12</i> : Review of Daily Monitoring Form Ψ <i>Format 13</i> . Identify <i>Scheduling Activities</i> (for clinician and consultant) Ψ <i>Format 11</i> . Daily Monitoring Form		
ACTIVITY SCHEDULING:			
1.	Present the summary of the previous session and the objectives of the current session.		5 min
2.	Review Weekly Task: <i>Daily Monitoring Log</i> → <i>Format 12</i> - If necessary, assess the difficulties in filling out the form and present options for solving them. If you have not been able to complete the form, take a few minutes to complete the column corresponding to the previous day or two so that you can work on it during the session. - Note the types of activities you are doing and whether they are pleasurable, important, both, or neither. Notice what activities you do too much of or do too little. - Identify the relationship between activities and moods and emotions, experiences of mastery and pleasure.		15 min
3.	Identify Activities to be Programmed → <i>Format 13</i> - Explain that the objective is the evaluation of activities that are part of an action plan that interrupts the cycle of maintenance of suffering, and then can increase the quality of life. The follow-up of this action plan is the heart of the activation. - Explain that, to be successful, plenty of sources should be explored to help generate a plan that is varied, concrete and focused on behavioral changes that can begin to be implemented immediately. It will be important to generate a generous amount of activities, as they can be referred back to whenever ideas are needed to program new activities.		35 min
4.	Session summary and closing task: completing the activity hierarchy		5 min
WEEKLY ACTIVITIES	Ψ Add, if necessary, Activities to be Scheduled → <i>Format 13</i> Ψ Complete the Daily Monitoring Form → <i>Format 12</i> Monitor moods, level of enjoyment, and importance		

Form 12
Review Daily Monitoring Form

REVIEWING DAILY MONITORING FORM

Next, we will review the daily monitoring sheets, paying attention to some aspects that may provide relevant information for treatment.

- Look into your monitoring of the previous week to see what types of activities you are doing and whether they are pleasurable, important, both, or neither. Often, people with depression spend little time on pleasurable activities; they also often withdraw from activities that are important to them. See if this is happening in your case.

- Is there variety in your activities or are the same ones always repeated?

- Considering your Enjoyment and Importance scores, if you were to increase the frequency of only one of the activities listed there, which one would it be?

- Considering your Enjoyment and Importance scores, if you were to reduce the frequency of just one of the activities that are there, what would it be?

- Notice what you are doing each day and whether these activities are leading you to feel better or worse.

Format 13
Identifying Activities to be Scheduled / Consultant

- The objective is the evaluation of activities that can be part of an action plan that interrupts the cycle of maintenance of suffering, and then can provoke a spiral of quality of life. Following this action plan is the heart of what we call “activating.”
- The key to success is to explore plenty of sources to help you generate a varied, concrete and focused plan for behavioral changes that you can begin to implement immediately. It is important to generate a generous amount of activities, as these will act as a safe place to return to whenever you need ideas for new activities.

1. Identify activities that were part of your **PERSONAL CARE** routine that have been neglected.

You can be guided by the following questions: Which of these routines have been interrupted or changed since you experienced this emotional pain, sleep routines, eating habits, exercise, daily hygiene, work schedules.
What activities could you start doing to resume these habits?

2. Identify activities that YOU USED TO ENJOY DOING BUT HAVE STOPPED DOING:

You can be guided by the following questions: What activities did you enjoy doing before you experienced this emotional pain, Could you help me understand the activities that made up your life before you became depressed or fell prey to depression, Are there any activities that you have stopped doing since you dislocated this emotional ankle?

3. Identify new activities that you WANT TO START DOING:

You can guide yourself with the following questions: Now that you are faced with the possibility of a new beginning, what kind of things would you like to start doing again? What would you be doing with your life if you weren't afraid to put that ankle down?

4. Identify the activities you **HAVE BEEN AVOIDING DOING** to avoid your emotional discomfort.

You can be guided by the following questions: Have you noticed yourself avoiding doing things?, When you give up, in what way do you do it?, How often do you notice yourself avoiding or postponing activities?, Are there things you would like to be doing, but they feel too difficult or too overwhelming?, How do you compensate when your ankle hurts?

5. Identify activities that would lead you to **ACHIEVE LONG-TERM GOALS** (values, aspirations, objectives):

You can be guided by the following questions: What long-term goals are important to you?, What is important to do in your life?, What kind of activities give meaning to your life?, What made you feel you had a purpose in your life?, What matters most to you?, What is at stake in your life right now?

Format 13
Identifying Activities to be Programmed / Clinician

- The objective is the evaluation of activities that can form part of an action plan that interrupts the cycle of maintenance of suffering and can then trigger a spiral of quality of life. The follow-up of this action plan is the heart of what we call “getting active”.
- The key to success is to explore plenty of sources to help you generate a varied, concrete and focused plan for behavioral changes that you can begin to implement immediately. It is important to generate a generous amount of activities, as these will act as a safe place to return to whenever you need ideas for new activities.

6. Identify activities that were part of your **PERSONAL CARE** routine that have been neglected.

You can be guided by the following questions: Which of these routines have been interrupted or changed since you experienced this emotional pain, sleep routines, eating habits, exercise, daily hygiene, work schedules.
What activities could you start doing to resume these habits?

7. Identify activities that YOU USED TO ENJOY DOING BUT HAVE STOPPED DOING:

You can be guided by the following questions: What activities did you enjoy doing before you experienced this emotional pain, Could you help me understand the activities that made up your life before you became depressed or fell prey to depression, Are there any activities that you have stopped doing since you dislocated this emotional ankle?

8. Identify new activities that you WANT TO START DOING:

You can guide yourself with the following questions: Now that you are faced with the possibility of a new beginning, what kind of things would you like to start doing again? What would you be doing with your life if you weren't afraid to put that ankle down?

9. Identify the activities you **HAVE BEEN AVOIDING DOING** to avoid your emotional discomfort.

You can be guided by the following questions: Have you noticed yourself avoiding doing things?, When you give up, in what way do you do it?, How often do you notice yourself avoiding or postponing activities?, Are there things you would like to be doing, but they feel too difficult or too overwhelming?, How do you compensate when your ankle hurts?

10. Identify activities that would lead you to **ACHIEVE LONG-TERM GOALS** (values, aspirations, objectives):

You can be guided by the following questions: What long-term goals are important to you?, What is important to do in your life?, What kind of activities give meaning to your life?, What made you feel you had a purpose in your life?, What matters most to you?, What is at stake in your life right now?

Format 11
Daily Monitoring Form

HOUR	Day:	D	I	Day:	D	I	Day:	D	I	Day:	D	I
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Overall Mood:				Overall Mood:			Overall Mood:			Overall Mood:		

Format 11
Daily Monitoring Form

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Overall Mood:				Overall Mood:				Overall Mood:				Overall Mood:	

DESCRIPTION SESSION 5

Reyes-Ortega & Zapata-Téllez, 2021

SESSION: 5		PHASE: Activity Scheduling	DURATION: 60 min
OBJECTIVE:		Review daily monitoring form, make a hierarchy of the activities to be scheduled, identify chosen rewards, schedule activities (simple activation)	
MATERIALS:		Ψ <i>Format 11</i> : Daily Monitoring Form Ψ <i>Format 12</i> : Review of Daily Monitoring Form Ψ <i>Format 14</i> . Hierarchy of Activities (for the clinician and consultant) Ψ <i>Format 15</i> . Rewards chosen (for the clinician ad consultant)	
ACTIVITY SCHEDULING:			
1. Present the summary of the previous session and the objectives of the current session.		5 min	
3. Review Weekly Task: <i>Daily Monitoring Log</i> → <i>Format 12</i> - If necessary, assess the difficulties in filling out the form and present options for solving them. If you have not been able to complete the form, take a few minutes to complete the column corresponding to the previous day or two so that you can work on it during the session. - Note the types of activities you are doing and whether they are pleasurable, important, both, or neither. Notice what activities you do too much of or do too little. - Identify the relationship between activities and moods and emotions, experiences of mastery and pleasure.		10 min	
4. Hierarchy of activities → <i>Format 13</i> Explain		35 min	
4. Activity Scheduling			
5. Chosen rewards			
6. Session summary and closing task: completing the activity hierarchy		5 min	
WEEKLY ACTIVITIES	Ψ Add, if necessary, Chosen rewards → <i>Format 15</i> Ψ Complete the Daily Monitoring Form → <i>Format 12</i> Monitor moods, level of enjoyment, and importance		

Format 14
Selection and Hierarchy of Activities / Consultant

WHAT IS IT FOR?

The objective is to select the activities that we will plan during the sessions. Since Format 13. Identifying Activities to Plan has many activities, we will select only a few of the most important ones to start doing, and write them in this format.

HOW IS IT DONE?

1. Look for Format 13. *Identifying Activities to Schedule* that you have completed and select the 3 most important activities in each area (self-care activities, activities you enjoyed doing but have stopped doing, new activities you want to start doing, activities you have been avoiding doing, and activities that lead to long-term goals) and list them below, in no particular order. Those will be the activities that we will begin to include in your life, so remember: activities should be observable, measurable, and in their smallest steps.
2. Next, number those activities in order of importance, with 1 being the most important to accomplish, and 15 being the least important to accomplish. One way to do this is to first identify the most important and assign it a 1, then identify the least important and assign it a 15, then rank the rest of the activities. We call this “building a hierarchy”.

The hierarchy will help us to start moving forward, planning the most important activities and then, gradually including the less important ones; but for the moment do not try to change anything, as we will dedicate ourselves to planning during the sessions.

NOTE:

To the extent that your daily activities are linked to what is important in your life, you are more likely to experience activities as both pleasurable and meaningful, and this will increase your sense of living the life you want to live.

[illegible]

Format 14
Selection and hierarchy of activities / Clinician

WHAT IS IT FOR?

The objective is to select the activities that we will plan during the sessions. Since Format 13. *Identifying Activities to Plan* has many activities, we will select only a few of the most important ones to start doing, and write them in this format.

HOW IS IT DONE?

1. Look for Format 13. *Identifying Activities to Schedule* that you have completed and select the 3 most important activities in each area (self-care activities, activities you enjoyed doing but have stopped doing, new activities you want to start doing, activities you have been avoiding doing, and activities that lead to long-term goals) and list them below, in no particular order. Those will be the activities that we will begin to include in your life, so remember: activities should be observable, measurable, and in their smallest steps.
2. Next, number those activities in order of importance, with 1 being the most important to accomplish, and 15 being the least important to accomplish. One way to do this is to first identify the most important and assign it a 1, then identify the least important and assign it a 15, then rank the rest of the activities. We call this “building a hierarchy”.

The hierarchy will help us to start moving forward, planning the most important activities and then, gradually including the less important ones; but for the moment do not try to change anything, as we will dedicate ourselves to planning during the sessions.

NOTE:

To the extent that your daily activities are linked to what is important in your life, you are more likely to experience activities as both pleasurable and meaningful, and this will increase your sense of living the life you want to live.

Format 15
Activity Scheduling and Hierarchy / Consultant

WHAT IS IT FOR?

The objective is that you can plan for the week some activities that you have already identified in your Activity Selection and Hierarchy.

HOW IS IT DONE?

1. From your activities form Format 14 *Hierarchy of Activities*, identify 2- 3 concrete activation tasks that you would like to start with this week (ideally, you should feel challenged but not overwhelmed). Some activities may be repeated more than once during the week.
 - a. Once you have selected them, break them down in this format: In the first row write the number you assigned to that activity according to your selection format and activity hierarchy. In the second row write which activation area that activity belongs to. In the third row write briefly why it is important to you. In rows 4 to 9 write the specific activity you plan to carry out, as well as when, at what time, how much time you will spend on it, where you will carry it out and what you need to do it. Finally, in row 10 write down the level of mastery you experience in carrying out the activity.
2. Then, on your blank Daily Monitoring form, write down the activity on the day(s) and time you plan to do it (for example, if you plan to “play with my daughter” write down that activity at a specific time, such as Monday at 8 a.m.). By thinking realistically about where you can fit the activity into your schedule you will find that you are more likely to be able to accomplish it.

During the week:

- Complete the form as you have been doing each day, but when you get to the box for the planned activity circle that box if you were able to do it. Remember to assign an enjoyment and importance score so that we can know if you experienced that activity the way you originally intended.
- If you did not complete the activity at the scheduled time, cross it out with a line (without deleting it) and write the activity you did instead. If possible, reschedule the activity for another time that week (or even that day) and circle it when you have completed it.

SOME CONSIDERATIONS:

- Schedule specific and concrete activities.
- Consider the difficulty of the tasks. If it is unrealistic to achieve your activities, break them down into smaller specific and achievable units.
- Adopt an experimental approach, there's no warranty that these activities will have a positive effect in your emotional pain, you need to carry them out to know.
- When doing your activities, remember that it is better to start an activity and leave it halfway through than to avoid doing it for fear of not completing it perfectly.
- Generate new routines. When you notice that an activity has a positive effect on your quality of life, make it part of your routine!

Activity Number (Hierarchy)			
What do I want to approach to this week? (Activation area)			
Why is this activity important to me?			
What do I have to do? (Concrete Action)			
¿When?			
¿Time?			
How much time will I devote to it?			
¿Where?			
What do I need?			
How capable do I feel of carrying it out? (0 to 100)			
What obstacles can arise?			
How do I overcome this obstacle?			

Format 15
Schedule Activities / Clinician

WHAT IS IT FOR?

The objective is that you can plan for the week some activities that you have already identified in your Activity Selection and Hierarchy.

HOW IS IT DONE?

1. From your activities form Format 14 *Hierarchy of Activities*, identify 2- 3 concrete activation tasks that you would like to start with this week (ideally, you should feel challenged but not overwhelmed). Some activities may be repeated more than once during the week.
 - a. Once you have selected them, break them down in this format: In the first row write the number you assigned to that activity according to your selection format and activity hierarchy. In the second row write which activation area that activity belongs to. In the third row write briefly why it is important to you. In rows 4 to 9 write the specific activity you plan to carry out, as well as when, at what time, how much time you will spend on it, where you will carry it out and what you need to do it. Finally, in row 10 write down the level of mastery you experience in carrying out the activity.
2. Then, on your blank Daily Monitoring form, write down the activity on the day(s) and time you plan to do it (for example, if you plan to “play with my daughter” write down that activity at a specific time, such as Monday at 8 a.m.). By thinking realistically about where you can fit the activity into your schedule you will find that you are more likely to be able to accomplish it.

During the week:

- Complete the form as you have been doing each day, but when you get to the box for the planned activity circle that box if you were able to do it. Remember to assign an enjoyment and importance score so that we can know if you experienced that activity the way you originally intended.
- If you did not complete the activity at the scheduled time, cross it out with a line (without deleting it) and write the activity you did instead. If possible, reschedule the activity for another time that week (or even that day) and circle it when you have completed it.

SOME CONSIDERATIONS:

- Schedule specific and concrete activities.
- Consider the difficulty of the tasks. If it is unrealistic to achieve your activities, break them down into smaller specific and achievable units.
- Adopt an experimental approach, there's no warranty that these activities will have a positive effect in your emotional pain, you need to carry them out to know.
- When doing your activities, remember that it is better to start an activity and leave it halfway through than to avoid doing it for fear of not completing it perfectly.
- Generate new routines. When you notice that an activity has a positive effect on your quality of life, make it part of your routine!

Activity Number (Hierarchy)			
What do I want to approach to this week? (Activation area)			
Why is this activity important to me?			
What do I have to do? (Concrete Action)			
¿When?			
¿Time?			
How much time will I devote to it?			
¿Where?			
What do I need?			
How capable do I feel of carrying it out? (0 to 100)			
What obstacles can arise?			
How do I overcome this obstacle?			

Format 11
Daily Monitoring Form

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Format 11
Daily Monitoring Form

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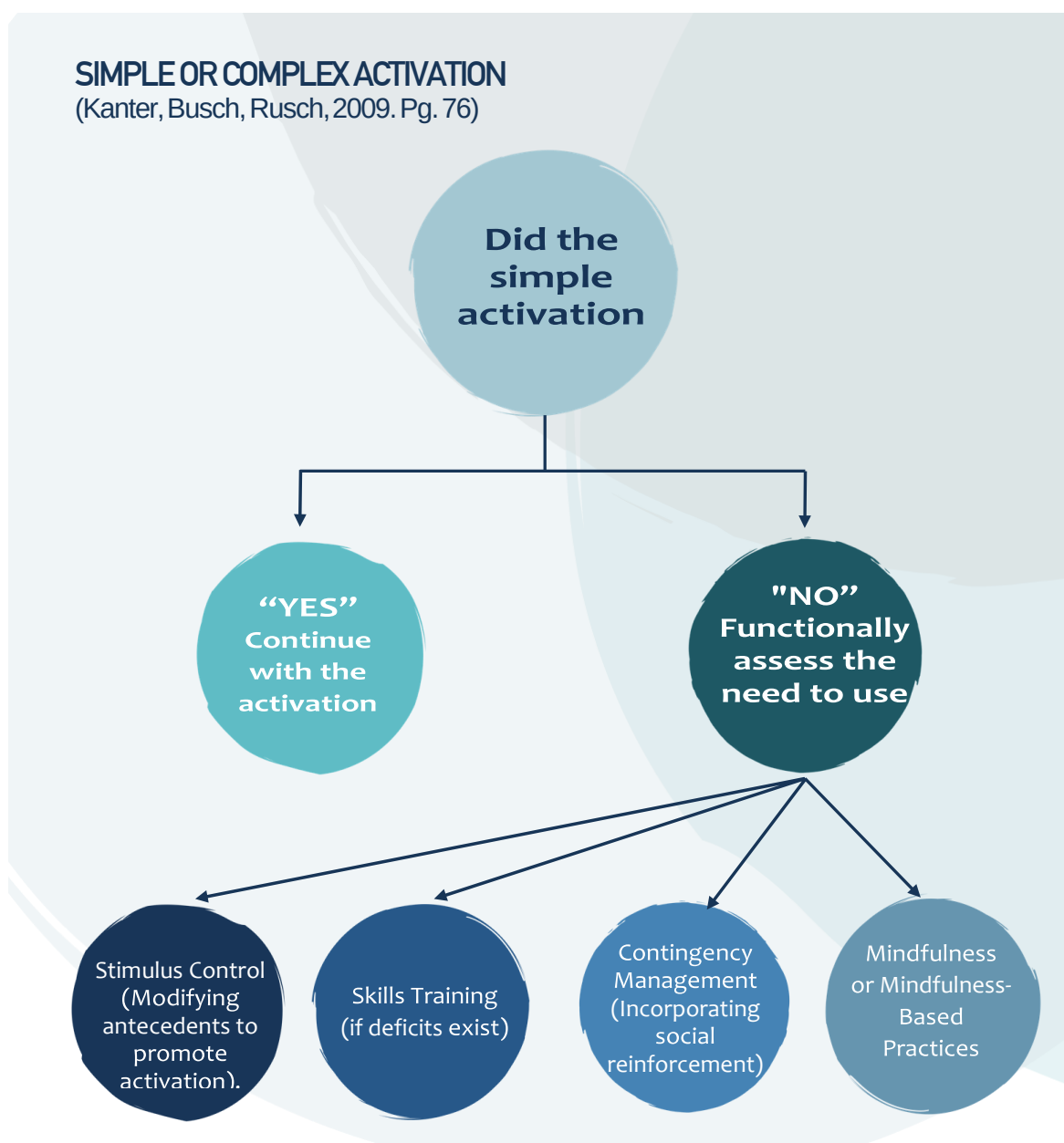
DESCRIPTION SESSION 6-8

Reyes-Ortega & Zapata-Téllez, 2021

SESSION: 6-8		PHASE: Scheduling Activities / Dealing with Avoidance	DURATION: 60 minutes
OBJECTIVE:		Review daily monitoring form and scheduled activities, present the TRAP model, and troubleshoot (if necessary), create a list of chosen rewards, and schedule activities.	
MATERIALS:		Ψ Format 11: Daily Monitoring Form Ψ Format 14: Hierarchy of Activities (Session 5) Ψ Format 15: Scheduling Activities (for clinician and client) Ψ Format 16: Celebrating Activation (if necessary) Ψ Format 17: TRAP Model (if necessary) Ψ Format 18: Chosen Rewards List (for clinician and client)	
ACTIVITY SCHEDULING:			
1. Present the summary of the previous session and the objectives of the current session.		5 min	
2. Review the weekly assignment: <i>Daily Monitoring Record</i> - If necessary, assess difficulties with completing it and present options to resolve them. If the form couldn't be completed, take a few minutes to fill out the column for the corresponding day or two days prior to work during the session. - Note the types of activities being performed and whether they are pleasurable, important, both, or neither. Notice which activities are being done more or less. - Identify the relationship between activities and mood states and emotions, experiences of mastery, and pleasure.		10 min	
3. Review the Scheduled Activity:			
<i>The simple activation WAS successful.</i> Format 16	- Celebrate the importance of having carried out the activation plan and discuss a way to incorporate activities that have had a positive effect on the client's psychological well-being into their daily routine. - Review the activities reviewed through the following questions: <i>How easy or difficult did you find them? How pleasurable and important did they seem? How did you feel about completing those activities? Would you like to continue with those activities, or would you prefer to select different ones? Did you notice any change in your mood? If not, consider that these changes may take some time.</i> - Move on to Activity 4 of the session.		10 min

<i>The simple activation WAS NOT successful - Format 17</i>	• Point out that it is normal and common to happen at the beginning of the path. • Explain the principles for addressing problems encountered in the activation plan using the TRAP model. • Having evaluated what occurs, attempt to resolve it for the following week. The most common difficulties are: 1. Antecedent problems (forgetfulness) → Implement stimulus control by creating reminders or removing distractions that may interfere with remembering scheduled activities. 2. Problems with skill deficits → Provide guidance and practice in social skills, problem-solving skills, or break down the activity into smaller steps so that it can be solved more easily. 3. Problems with observable consequences (other people discourage the completion of scheduled activities) → Implement contingency management by teaching to set boundaries or asking others for moral support to carry out the activities. If necessary, interview key individuals with whom they live to manage how they can be incorporated as members of the Behavioral Activation team. 4. Problems with private events (emotions, thoughts) → Mindfulness-based activation to intentionally pay attention to what is happening in the present moment, with detachment from the thoughts and sensations it contains, freeing oneself from the tendency to respond with obedience or fight against feelings. • Move on to Activity 5 of the session. • Explain Activity 4 as a Homework Assignment.	journey. 20 min
4. Create a list of chosen rewards. → Format 18 • List and rank 10 "rewards" that the client can access each week for completing the scheduled activity (token economy). • If the client completed the scheduled activities during the current week, they will be able to access the first identified reward.		15 min
5. Scheduling Activities → Format 15 • Schedule past activities to be repeated and enter them into the daily monitoring form. • Identify 2-3 specific activation tasks for this week and break them down. Then, in the blank Daily Monitoring Form, write down the activity on the day(s) and time it is planned to be carried out.		15 min

6. Session summary and closing	5 min
WEEKLY ACTIVITIES:	Ψ Complete the Daily Monitoring Form→Format 11 • Complete the form as you have been doing each day, but when you reach the planned activity box, circle around that box if you have been able to complete it. Remember to assign a score for enjoyment and importance. • If the activity wasn't completed at the scheduled time, cross it out with a line (without erasing it) and write down the activity that was done instead. If possible, reschedule the activity for another time later in the week (or even on the same day), and circle it when it's completed.



Format 16

Celebrating activation

Having done the activation plan was very important!, now you should look for ways to make the activities that have had a positive effect on your psychological well-being part of your daily routine.

Discuss with your psychotherapist the following questions:

How easy or difficult were they for you?

How pleasant and important did you find them?

How did you feel about having accomplished those activities?

Would you like to continue with these activities or would you prefer to select different ones?

Did you notice any changes in your mood?, if not, consider that these changes may take some time.

Format 17

The Tramp Model

It is very easy to be tempted to judge yourself when difficulties arise in maintaining the Activation plan. Instead of resorting to character explanations such as “I’m lazy” or “I have a tendency to sabotage myself”, your psychotherapist will help you maintain an attitude consistent with Behavioral Activation, at all times acting as a sports coach to help you maintain a mindset as active as your actions.

PRINCIPLES FOR RESPONDING TO THE PROBLEMS ENCOUNTERED IN THE ACTIVATION PLAN

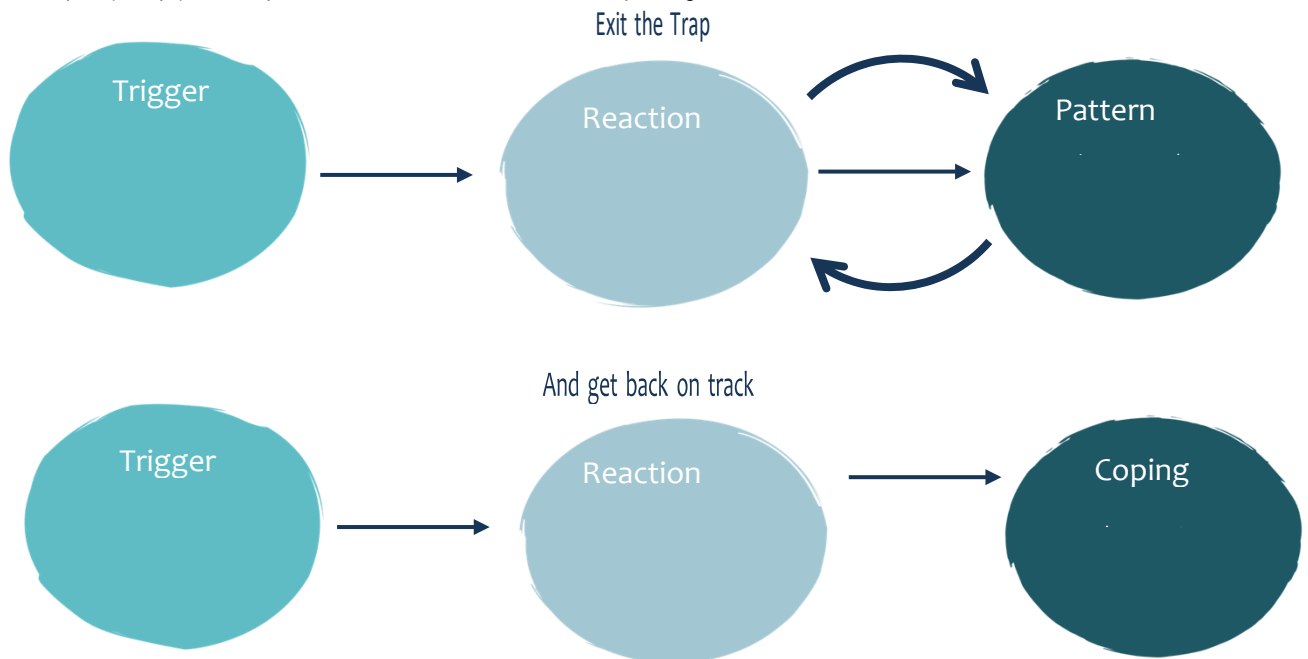
1	Distinguish triggers, responses, and avoidance patterns that got in the way of activation.
2	Recognize that your avoidance responses and patterns are natural, understandable, and common. Avoid punishing yourself, “punishment does not help you understand avoidance, nor does it point to the solution”.
3	Identify and discuss with your psychotherapist the short-term benefits of avoidance patterns and identify the long-term problems they create. Recognize that avoidance patterns can be active or passive.
4	Remember that “Behavioral Activation” is a conditioning plan that requires action, even when you don't feel like acting.
5	Connect the activity to be programmed with the goals and values you have in your life, both in the short and long term.
6	Plan your activities this week, concretely and anticipating the emergence of the temptation to avoid.

GET OUT OF THE TRAP AND BACK ON THE TRAIL

The acronym TRAP, derives from *Trigger + Response + Avoidance Pattern*.

- **T** rigger, refers to the negative events that occurred when you were about to do your scheduled activity, or even before that, and that discouraged you from staying active.
- **R** esponse, shows the emotional distress elicited by the Trigger.
- **A** voidance **P** attern, designates the avoidance strategy you adopted to decrease, avoid or escape of the emotional distress experienced. The avoidance pattern maintains the experience of suffering because it prevents coping with the events that triggered it.

With the help of your psychotherapist, write down the information corresponding to each of the items described above.



To get out of this trap, you can keep the acronym TRAC in mind, which is derived from Trigger + Response + Alternative Coping. The new element with respect to TRAP is **Alternative Coping**, which indicates a new action pattern that interrupts the established avoidance pattern and promotes the performance of the scheduled activity (or activities).

Format 18
List of Chosen Rewards / Consultant

Chosen Rewards:

Ψ _____

Ψ _____

Ψ _____

Ψ _____

Ψ _____

Ψ _____

Ψ _____

Ψ _____

Ψ _____

Ψ _____

Week	Chosen awards	Accomplished?	
		Yes	No
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2			
3			
4			
5			
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7			
8			
9			
10			

Format 18
List of Chosen Rewards / Clinician

Ψ _____

Ψ _____

Ψ _____

Ψ _____

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Ψ _____

Week	Chosen awards	Accomplished?	
		Yes	No
1			
2			
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Format 15
Schedule Activities / Consultant

Activity number (hierarchy)			
What do I want to approach this week? (Activation area)			
Why is this activity important to me?			
What do I have to do? (Concrete action)			
When?			
At what time?			
How much time will I spend on it?			
Where?			
What do I need?			
How capable do I feel to carry it out? (0 al 100)			
What obstacle may arise?			
How do I overcome this obstacle?			

Format 15
Schedule Activities / Clinician

Activity number (hierarchy)			
What do I want to approach this week? (Activation area)			
Why is this activity important to me?			
What do I have to do? (Concrete action)			
When?			
At what time?			
How much time will I spend on it?			
Where?			
What do I need?			
How capable do I feel to carry it out? (0 al 100)			
What obstacle may arise?			
How do I overcome this obstacle?			

Format 11
Daily Monitoring Form

HOURL	Day:	D	I	Day	D	I	Day:	D	I	Day:	D	I			
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Overall Mood:				Overall Mood:				Overall Mood:				Overall Mood:			

Format 11
Daily Monitoring Format

HOURL	Day	D	I	Day:	D	I	Day:	D	I	Day:	D	I			
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Overall Mood:				Overall Mood:				Overall Mood:				Overall Mood:			

SESSION 9 DESCRIPTION

Reyes-Ortega & Zapata-Téllez, 2021

SESSION: 9	PHASE: Activity Scheduling / Deal with Avoidance	DURATION: 60 min
OBJECTIVE:	Review daily monitoring form and scheduled activities, present TRAP model and troubleshoot (if necessary), agree on chosen weekly reward (if applicable), schedule activities, and initiate relapse plan (if there is time during the session).	
MATERIALS:	Ψ <i>Format 11: Daily Monitoring Form</i> Ψ <i>Format 14: Hierarchy of Activities (Session 5)</i> Ψ <i>Format 15: Schedule activities (for clinician and consultant)</i> Ψ <i>Format 17: The TRAP model (if necessary)</i> Ψ <i>Format 18: List of Chosen Rewards (Session 6)</i> Ψ <i>Format 19: Guidelines for staying active</i>	
ACTIVITY SCHEDULING:		
1. Present the summary of the previous session and the objectives of the current session.		5 min
2. Review Weekly Task: Daily Monitoring Record		5 min
3. Review the Scheduled Activity:		
Simple activation WAS successful	• Celebrate the importance of having completed the activation plan and discuss a way to make the activities that have had a positive effect on the client's psychological well-being as part of his or her daily routine. • Agree on obtaining the chosen weekly reward → Format 18	10 min
Simple activation was NOT successful Format 17	• Explain the principles for responding to problems encountered in the activation plan using the TRAP model. • Evaluate what happened, try to solve it for the following week. The most frequent difficulties are: 1. Antecedent problems (forgetfulness) → Do stimulus control. 2. Problems with skills deficits → Orient and rehearse social skills, problem solving skills, or segment the activity into smaller steps so that it can be solved more easily. 3. Problems with observable consequences (other people discourage completion of planned activities) → Do contingency management 4. Problems with private events (emotions, thoughts) → Activation based on mindfulness.	15 min

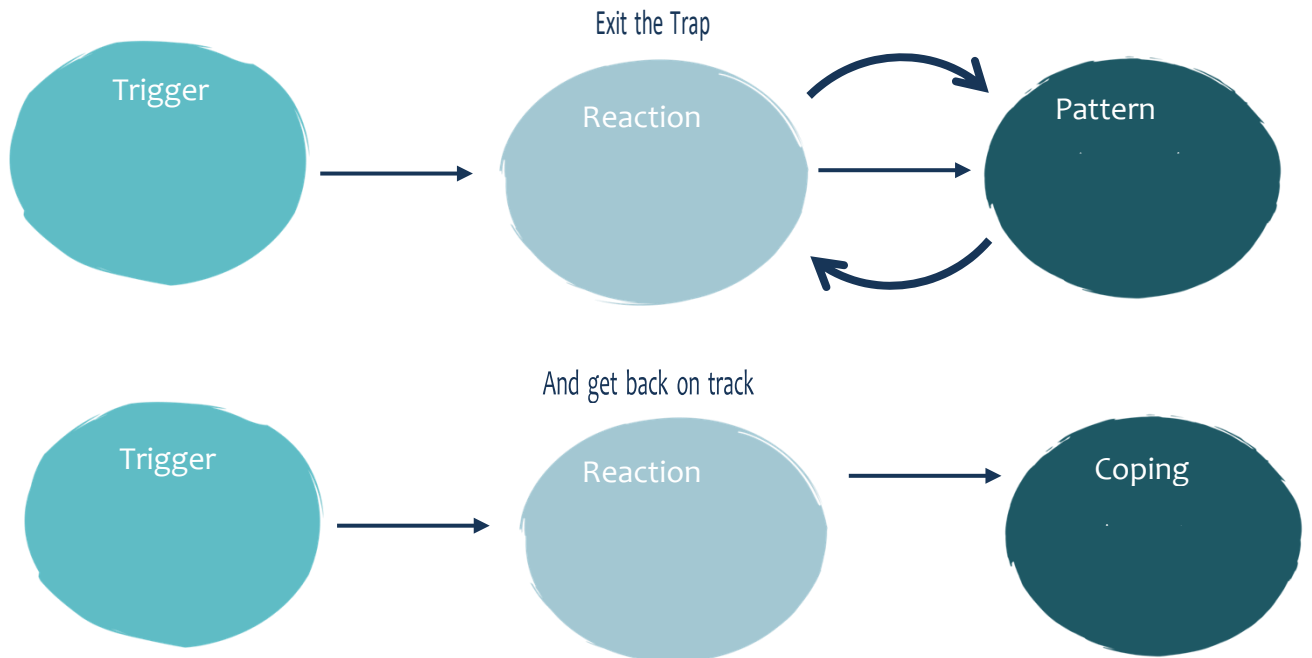
4. Schedule Activities → Format 15 - Schedule past activities so that they can be repeated and schedule them on the daily monitoring form. - Identify 2 - 3 specific activation tasks for this week and break them down. Subsequently, on the blank Daily Monitoring form, write down the activity on the day(s) and time(s) when you plan to perform them.		10 min
5. Initiate the development of a relapse plan → Format 19 - Summarize what, so far, the client has learned to help him/her feel better and live in a healthier way if he/she begins to feel depressed again: 1) He/she has learned to observe the impact that his/her daily activities have on his/her mood and life in general, 2) He/she has learned to consider the activities that are valuable, prioritized and important, 3) He/she has learned to create a hierarchy for activities and to include them effectively in his/her life, and 4) He/she has learned to solve the problems that arise in carrying them out. - Identify contexts that predispose to relapse; create a list of what you start doing more of and what you start doing less of when you start to feel depressed; identify resources you can implement when you notice yourself in any of these contexts or engaging in any of these behaviors; create a list of activities you enjoy doing when you are not feeling depressed; create a list of activities that are important even if you do not enjoy doing them; identify a list of obstacles you may encounter and how to overcome them.		Use remaining time, if any
4. Session summary and closing		5 min
ACTIVITIES FOR THE WEEK:	Ψ Complete the Daily Monitoring Form → Format 11 - Complete the form as you have been doing each day, but when you get to the box for the planned activity, circle that box if you were able to complete it. Remember to assign a score for enjoyment and importance. - If the activity was not completed at the scheduled time, cross it out with a line (without erasing it) and write the activity that was completed in its place. If possible, then re-plan the activity for another time later in the week (or even on the same day) and circle it when it has been completed.	

The TRAMP Model / Get out of the Tramp and back on Track

The acronym TRAP, derives from *Trigger + Response + Avoidance Pattern*.

- **T** rigger, refers to the negative events that occurred when you were about to do your scheduled activity, or even before that, and that discouraged you from staying active.
- **R** esponse, shows the emotional distress elicited by the Trigger.
- **A** voidance **P** attern, designates the avoidance strategy you adopted to decrease, avoid or escape of the emotional distress experienced. The avoidance pattern maintains the experience of suffering because it prevents coping with the events that triggered it.

With the help of your psychotherapist, write down the information corresponding to each of the items described above.



To get out of this trap, you can keep the acronym **TRAC** in mind, which is derived from Trigger + Response + Alternative Coping. The new element with respect to TRAP is **Alternative Coping**, which indicates a new action pattern that interrupts the established avoidance pattern and promotes the performance of the scheduled activity (or activities).

Format 15
Schedule Activities / Consultant

Activity number (hierarchy)			
What do I want to approach this week? (Activation area)			
Why is this activity important to me?			
What do I have to do? (Concrete action)			
When?			
At what time?			
How much time will I spend on it?			
Where?			
What do I need?			
How capable do I feel to carry it out? (0 al 100)			
What obstacle may arise?			
How do I overcome this obstacle?			

Format 15
Schedule Activities / Clinician

Activity number (hierarchy)			
What do I want to approach this week? (Activation area)			
Why is this activity important to me?			
What do have to do? (Concrete action)			
When?			
At what time?			
How much time will I spend on it?			
Where?			
What do I need?			
How capable do I feel to carry it out? (0 al 100)			
What obstacle may arise?			
How do I overcome this obstacle?			

Format 19

Guide to Staying Active

At this point you have learned a number of skills that can help you feel better and live healthier if you start to feel depressed again. You have learned to observe the impact that your daily activities have on your mood and your life in general, you have learned to consider the activities that are valuable, priority and important to you, you have learned to create a hierarchy for activities and to include them accordingly, and effectively in your life, and you have learned to solve the problems that arise to carry them out.

You have learned that you can make changes in your life. These skills are not limited to treatment, but can help you along the way. Eventually, you may find that you are living consistently with valuable directions for your life, without having to use the forms, but you may also find that reviewing this format and the skills helps.

Of course, feelings of sadness and low energy may return, but depression is unlikely to persist when you live a healthy, meaningful, and fulfilling life.

When you look back at your first daily monitoring format, what do you notice?

PREVENTING RELAPSES

Feeling sad is not the same as depression. Sadness is a feeling, but depression occurs when we negatively alter our activity patterns. For this reason, it is important to identify your behavior patterns now, but also those at the beginning of treatment, which will help you be more attentive to them in the future.

Some ideas to avoid relapses are:

- Create a list of those things you start doing more when you start to feel depressed (for example, spending time in bed during the day). Also make a list of those things that you start doing less (for example, missing friends' gatherings). The proposal is that you can be careful not to repeat these behavior patterns in the future.

- It can also be very useful to record those activities that you have discovered that have high enjoyment and/or importance scores. Having them identified can help you plan them more frequently in times when your spirits decline.
- Consider that these activities that we carry out throughout the sessions can be seen as new skills that must continue to be practiced so that they become stronger.

We recommend that you return to your formats from time to time and review and edit them according to your current priorities. That can help you stay on track to develop healthy activities based on the things that are important in your life.

No matter what has happened in the past, it is possible to make changes in our lives, make the best of circumstances, and spend time doing activities that fill your life with purpose and meaning.

STAYING ON COURSE

CONTEXTS, SITUATIONS, OR EVENTS THAT PREDISPOSE ME TO A RELAPSE ARE:	

CONDUCTS THAT SHOW A POSSIBLE RELAPSE ARE:	
Things I do more frequently:	Things I do less frequently:

SOME RESOURCES THAT I CAN IMPLEMENT WHEN I NOTICE ANY OF THOSE CONTEXTS/SITUATIONS OR BEHAVIORS:	

THE ACTIVITIES I ENJOY DOING WHEN I'M NOT FEELING DEPRESSED ARE:	

ACTIVITIES THAT ARE IMPORTANT, EVEN IF I DON'T ENJOY DOING THEM ARE:	

THE OBSTACLES I MAY ENCOUNTER ARE:	

HOW CAN I OVERCOME SUCH OBSTACLES?	

Answer the following questions that can help you reflect on the most important aspects of your experience in this Behavioral Activation program. Your answers will serve as feedback for your psychotherapist. Once you answer them, you and your psychotherapist can reflect on them and use these answers as a reflection that honors the work you have done together and that serves as a farewell to your participation in the process.

- 1. What could you say to yourself to celebrate the results of your participation in these sessions of Behavioral Activation?

Format 11
Daily Monitoring Form

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Overall Mood:				Overall Mood:				Overall Mood:				Overall Mood:			

Format 11
Daily Monitoring Format

Hour	Day	D	I	Day:	D	I	Day:	D	I	Day:	D	I			
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Overall Mood:				Overall Mood:				Overall Mood:				Overall Mood:			

SESSION 10 DESCRIPTION

Reyes-Ortega & Zapata-Téllez, 2021

SESSION: 10		PHASE: ACTION/Relapse prevention plan /Closing	DURATION: 60 min
OBJECTIVE:	Review the scheduled activities, briefly remember the TRAP model (if necessary), agree on the chosen weekly reward (if applicable), prepare a Relapse Plan, present the ACTION acronym, review the consultant's experience in the behavioral activation program.		
MATERIALS:	Ψ <i>Format 11:</i> Daily Monitoring Form (for the consultant) Ψ <i>Format 15:</i> Activity Scheduling (for the consultant) Ψ <i>Format 18:</i> List of Chosen Rewards (Session 6) Ψ <i>Format 17:</i> The TRAMP Model (for the consultant) Ψ <i>Format 19:</i> Guide to Staying Active Ψ <i>Format 20:</i> ACTION Ψ <i>Format 21:</i> Behavioral Activation Experience Questionnaire		
ACTIVITY SCHEDULING:			
5. Present summary of the previous session and the objectives of the current session.			5 min
6. Review the Scheduled Activity:			
Simple Activation WAS successful	• Celebrate the importance of having carried out the activation plan and discuss a way to include the activities that have had a positive effect on the psychological well-being of the consultant into part of their daily routine. • Evaluate the reviewed activities using the following questions: How easy or difficult were they for you? How pleasant and important did they seem to you? How did you feel about having completed those activities? Would you like to continue with those activities? Would you prefer to modify them? Did you notice any changes in your mood? If not, consider that these changes may take some time. • Agree to obtain the chosen weekly reward → Format 18		10 min
Simple Activation WAS NOT successful Format 17	• Point out that it is normal and frequent for it to happen at the beginning of the route. • Briefly explain the principles for responding to the problems found in the activation plan using the TRAP model and ask them to establish and implement a solution as a home activity.		10 min

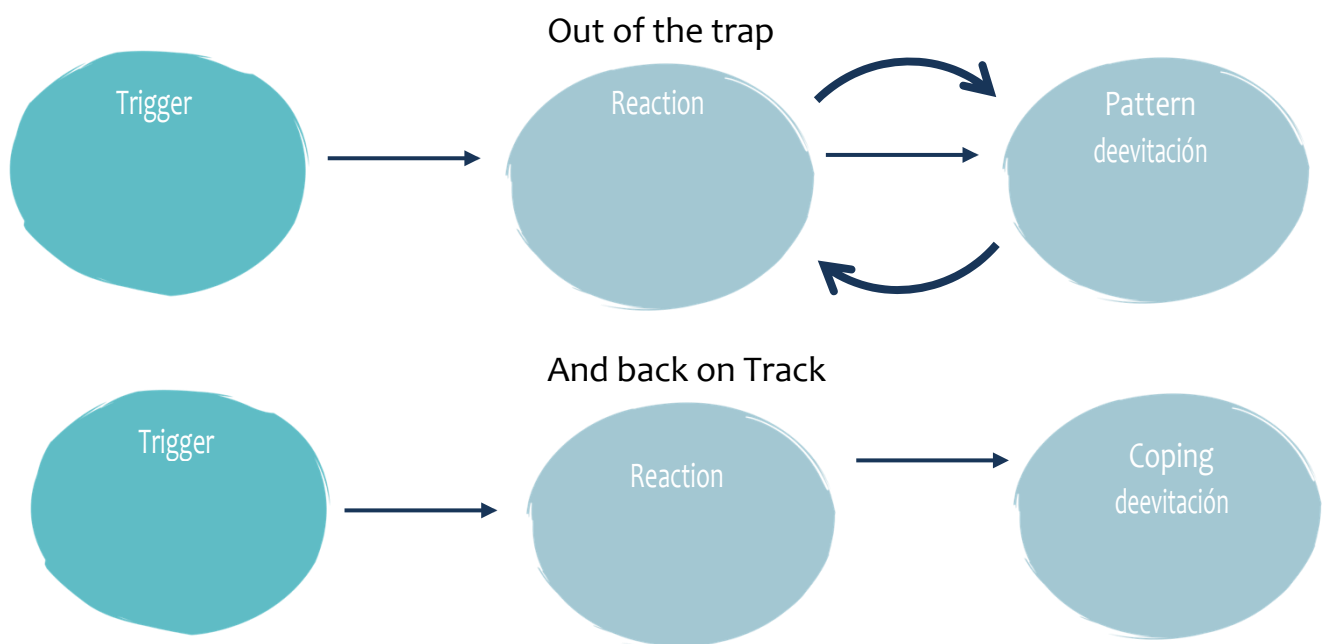
	7. Design a Relapse Prevention Plan → Format 19 - Summarize what, so far, the client has learned to help them feel better and live healthier if they begin to feel depressed again: 1) They have learned to observe the impact that their daily activities have on their mood and life in general, 2) You have learned to consider activities that are valuable, priority and important, 3) You have learned to create a hierarchy for activities and to include them effectively in your life, and 4) You have learned to solve the problems that they show up to carry them out. - Identify the contexts that predispose you to relapse. - Create a list of what you start doing more and what you start doing less when you start to feel depressed. - Identify resources that you can implement when you notice being in any of these contexts or carrying out any of these behaviors. - Create a list of activities you enjoy doing when you're not feeling depressed. - Create a list of activities that are important even if you don't enjoy doing them. - Identify a list of obstacles you may encounter and how to overcome them.	20 min
	8. Introduce the acronym ACTION as an aid to staying active → Format 19	10 min
	9. Session summary and closing	5 min
ACTIVITIES FOR THE WEEK:	10. Activity Scheduling → Format 15 - Schedule past activities so they can be repeated and schedule them on the daily monitoring form. - Identify 2 - 3 specific activation tasks for and break them down. Subsequently, on the blank Daily Monitoring form, write down the activity on the day (or days) and time in which you plan to carry it out.	

The TRAP Model / Getting Out of the Trap and Back on Track

The acronym TRAP o Trap is derived from Trigger + Response + Avoidance Pattern.

- Trigger refers to the negative events that occurred when you were about to do your scheduled activity, or even before them, that discouraged you from staying active.
- Response indicates the emotional distress elicited by the Trigger.
- Avoidance Pattern designates the avoidance strategy you adopted to diminish, avoid or escape from the emotional discomfort experienced. The avoidance pattern maintains the experience of suffering because it prevents you from facing the events that triggered it.

With the help of your psychotherapist, write down the information corresponding to each of the items described above.



To get out of the trap you can keep the acronym **TRAC** in mind, which derives from Trigger + Response + Alternative Coping. The new element with respect to TRAP is **Alternative Coping**, which indicates a new action pattern that interrupts the established avoidance pattern and promotes the performance of the programmed activity (or activities).

Format 15
Schedule Activities / Consultant

Activity number (hierarchy)			
What do I want to approach this week? (Activation area)			
Why is this activity important to me?			
What do have to do? (Concrete action)			
When?			
At what time?			
How much time will I spend on it?			
Where?			
What do I need?			
How capable do I feel to carry it out? (0 al 100)			
What obstacle may arise?			
How do I overcome this obstacle?			

ACTION



Assess

What is this behavior due to? What are its consequences? Are they depressants or activators? Is it consistent with my long-term goals?



Choose

Choose an action. What action do you choose?



Try

Put the chosen action into practice. Review your plan in detail to implement the new behavior.



Integrate

The new action in your habits. If you are trying something new and adopt a behavior that is contrary to your mood, you should do it more than once before confirming whether it is useful or not. Integrate it into your routines. How will you do it?



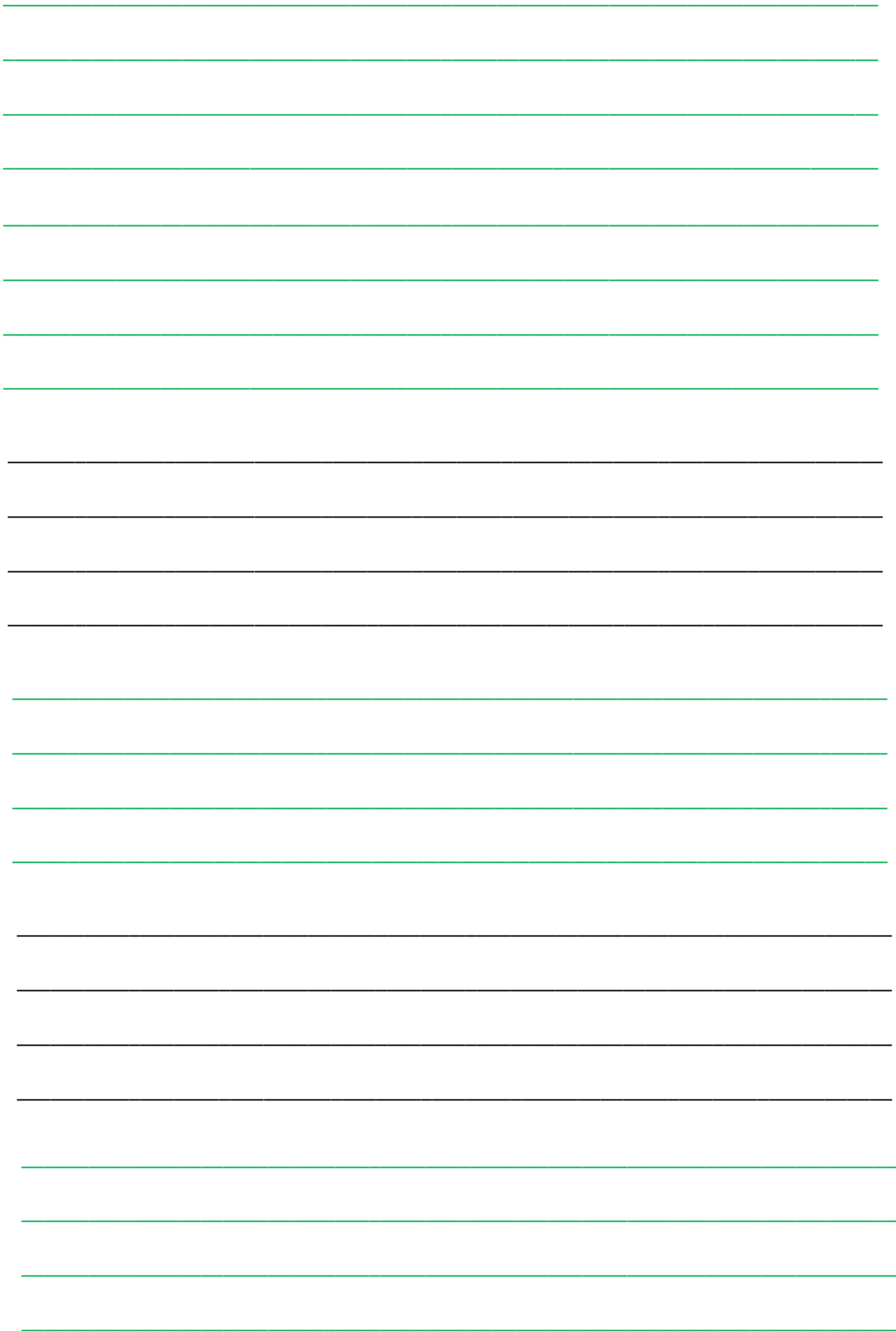
Observe

The results. How did it turn out? Do you feel worse or better after the action you chose to carry out? Have you gotten closer to any of your goals through this action? Have you integrated this behavior into your activity log as a routine? What changes have you noticed?



Never give up

Repeat the previous steps. Developing a new habit of activity and engagement requires repeated effort. Over time, these antidepressant behaviors will become automatic, even if you are discouraged.



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Daily Monitoring Form

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Guide to Staying Active

We recommend that you go back to your formats from time to time and review and edit them according to your current priorities. This can help you stay on track to develop healthy activities according to the things that are important in your life.

No matter what has happened in the past, it is possible to make changes in our lives, make the best of circumstances and spend time doing activities that fill your life with purpose and meaning.

KEEPING ON TRACK

THE CONTEXTS, SITUATIONS OR EVENTS THAT PREDISPOSE ME TO A RELAPSE ARE:

BEHAVIORS THAT SIGNAL A POSSIBLE RELAPSE ARE:

Things I am starting to do more frequently	Things I am starting to do less frequently

**Some resources I can implement when I notice any of these contexts/
situations or behaviors:**

THE ACTIVITIES I ENJOY DOING WHEN I AM NOT FEELING DEPRESSED ARE:

**THE ACTIVITIES THAT ARE IMPORTANT, EVEN IF YOU DON'T ENJOY DOING
THEM ARE:**

THE OBSTACLES I MAY ENCOUNTER ARE:	

HOW CAN I OVERCOME SUCH OBSTACLES?	